

Evidence for trustworthy premarital health education as primary prevention of intimate-partner harm

A systematic evidence map

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Claim boundary. This is a systematic evidence map (scoping review) of the literature searched and admitted while building the evidence library behind the Trustworthy Premarital Health Education (T-PHE) programme theory. It is *not* a systematic review of effectiveness, has no protocol registration (no PROSPERO record), used single-reviewer screening with an AI-assisted, human-owned adversarial verification pass rather than independent duplicate screening, applied no risk-of-bias tool at the individual-study level, and performed no meta-analysis. Nothing in this document is evidence that T-PHE works. The claim ceiling is the accepted TIMA/FIMA 2026 abstract and the T-PHE full-paper draft ([paper/main.pdf](#) in the public repository); this evidence map is a separate, subordinate document that maps what the literature search behind that programme theory actually found, admitted, and could not find.

Abstract

Background. Trustworthy Premarital Health Education (T-PHE) is a proposed governance architecture for premarital education, stated as seven testable propositions (P1–P7) about a decision-control mechanism, a private channel, cultural intelligibility, a stop rule, referral quality, the gap between conventional and safeguarding outcomes, and repairable records. Before any of these propositions is tested, the evidence already assembled while building them needs to be mapped, not narrated.

Methods. Eighteen category-specific, and to that extent structured, literature searches were run over PubMed (NCBI E-utilities) plus six supplementary reading-list passes, logged as 18 angle files. A single reviewer screened records; an independent AI-assisted verifier pass re-fetched each PubMed record and assigned a verdict of KEEP, FIX (admitted with a correction), DROP, or CONTEXT-ONLY (not PubMed-indexed, no evidential weight). No protocol was registered, no risk-of-bias tool was applied at study level, and no meta-analysis was performed. All counts below are computed directly from the logged search, verification, library, and knowledge-graph files by an accompanying script (`build_prisma.py`); none is estimated.

Results. The search logged 358 candidate records across 18 angle files (160 PubMed E-utilities queries), reducing to 277 unique PMIDs after de-duplication among the PMID-bearing candidates. The verifier processed 358 records: 250 KEEP, 97 FIX, 2 DROP, and 9 CONTEXT-ONLY. The library admits 347 category-level entries (313 unique PMIDs, plus 18 admitted non-PMID rows corresponding to 17 unique guidance documents without a PMID – one blank-identifier collision between two distinct WHO documents was resolved by a deterministic fallback key rather than a naive count of rows; see Limitations) across 18 categories, 330 unique records in total. Records linked in the knowledge graph to at least one of the seven propositions numbered 109 unique records, distributed unevenly: 44 linked to P1, 37 to P5, 23 to P4, 18 to P2, 16 to P3, and only 6 each to P6 and P7. No record in the library was PubMed-indexed as directly evaluating a premarital education programme, of any design, in a Thai or Thai Muslim population.

Conclusions. The evidence library supports the plausibility of a premarital governance layer and repeatedly documents a knowledge-to-behaviour and process-to-outcome gap in adjacent literatures (screening, relationship education, cultural-competence training), but it contains no direct effectiveness evidence for T-PHE or for any comparable programme, and coverage of Thai/Thai-Muslim samples, premarital-specific trials, validated decision-control instruments, and the two proposed extension layers (Layer 0, Layer 2) remains sparse to absent. This map is offered to pre-specify the scope of a future full systematic review with protocol registration, not as a substitute for one.

1 Introduction

1.1 Why a map, not a narrative

The T-PHE programme theory (full paper, `paper/main.pdf` in the public repository) states seven testable propositions and cites an evidence base assembled across 18 literature-search categories. That evidence base is narrated once, in prose, in the full paper’s Discussion and Appendix E. A narrative synthesis is not the same document as a map of what was searched, what was found, what was admitted, what was verified, and what gap each search pass itself logged. This document is that map: it reports the literature-mapping process as a scoping review in its own right, against the PRISMA extension for Scoping Reviews (PRISMA-ScR), and treats every count as something to be computed from the logged files, not recalled from the narrative.

1.2 Review questions

The seven propositions that organise the T-PHE programme theory are used here as the review questions, unchanged from the full paper:

- **P1.** A governance layer improves decision control beyond content-only education.
- **P2.** A routine private channel makes materially relevant concerns more visible.
- **P3.** Cultural intelligibility helps only when culture cannot override consent or safety.
- **P4.** A stop rule prevents routine joint education from worsening risk when fear or coercion appears.
- **P5.** Referral quality depends on timeliness, accessibility and follow-through, not offer alone.

- **P6.** Conventional programme success can coexist with T-PHE failure.
- **P7.** Repairability is necessary because relevant information emerges over time.

For each, Results §3.3 charts every record the knowledge graph links to that proposition, its design, its population, its finding as charted, and whether it supports, complicates, or is relevant only to the proposition (no record in the library is tagged *contradicts* any proposition; this absence is itself reported, not filled in).

2 Methods

2.1 Eligibility

Eligible records were PubMed-indexed literature (any design) or named global-guidance documents (WHO, USPSTF, NICE, Cochrane, MRC) relevant to one of 18 pre-declared search categories spanning IPV burden and prevention frameworks, premarital/relationship education, screening and referral, couple-based work and coercive control, decision-autonomy measurement, Muslim/Thailand/Southeast Asian populations, multicultural clinical safety, preconception medicine, global guidance, Thailand and the southern border provinces, premarital knowledge and well-being, culture as a health determinant, digital self-assessment, peer online support, assessor independence and check-in models, recognition of coercive control, humility/compassion/self-regulation, and tier-one Islamic medical ethics (library/library.json **categories**). Sources outside PubMed and the named global-guidance bodies were admitted only as *context*, carrying no evidential weight (library/library.json **context_sources**); two identified records were dropped outright as unverifiable (library/library.json **not_admitted**).

2.2 Information sources and search strategy

Twelve of the 18 categories were searched directly against PubMed using NCBI E-utilities (**esearch/efetch**), with every query string logged in that category’s **search-*.json** file under **queries_run**. Six categories (external-culture, external-dossier, external-draft, external-multicultural, external-thailand, external-wellbeing) were built from a supplied external reading list rather than a fresh PubMed search pass, and carry no **queries_run** log; this is stated plainly in Table 1, not smoothed over. The full set of logged query strings, reproduced verbatim, is in Table 1 (Supplementary search strategy).

Table 1: Search strategy: the exact PubMed E-utilities query strings logged per angle (**queries_run** in each **search-*.json** file). Six angle files (external-culture, external-dossier, external-draft, external-multicultural, external-thailand, external-wellbeing) were supplied as logged external reading lists rather than a **queries_run** search log, and carry no query strings to reproduce here; this is stated, not estimated.

Search angle	Query string
assessor-independenc e-checkin-models	(conflict of interest[Title/Abstract] OR independenc e[Title/Abstract]) AND (safeguarding[Title/Abstract] OR "child protection"[Title/Abstract] OR risk assessment[Title/Abstract]) AND (assessor[Title/Abstract] OR practitioner[Title/Abstract] OR provider[Title/Abstract]) ("intimate partner violence"[MeSH] OR "domestic violence"[MeSH]) AND ("risk assessment"[Title/Abstract]) AND (DASH[Title/Abstract] OR ODARA[Title/Abstract] OR "Danger Assessment"[Title/Abstract]) "warm handoff"[Title/Abstract] AND (referral[Title/Abstract] OR "care navigation"[Title/Abstract]) "brief contact"[Title/Abstract] AND (self-harm[Title/Abstract] OR suicide[Title/Abstract]) AND (intervention[Title/Abstract] OR postcard[Title/Abstract])

Table 1 continued

Search angle	Query string
	"single session intervention"[Title/Abstract] AND (prevention[Title/Abstract] OR mental health[Title/Abstract]) postpartum[Title/Abstract] AND "check-in"[Title/Abstract] AND (intervention[Title/Abstract] OR screening[Title/Abstract]) (fidelity[Title/Abstract]) AND ("prevention program"[Title/Abstract] OR "prevention programme"[Title/Abstract]) AND (incentive[Title/Abstract] OR completion[Title/Abstract]) ("dual relationship"[Title/Abstract] OR "conflict of interest"[Title/Abstract]) AND (clinician[Title/Abstract] OR therapist[Title/Abstract]) AND ethics[Title/Abstract] "dual relationship"[Title/Abstract] AND (assessment[Title/Abstract] OR evaluation[Title/Abstract]) AND (bias[Title/Abstract] OR objectivity[Title/Abstract]) - 0 results "independent assessor"[Title/Abstract] OR "independent reviewer"[Title/Abstract] AND (safeguarding[Title/Abstract] OR "risk assessment"[Title/Abstract]) - yielded off-topic results, not used "who administers"[Title/Abstract] AND ("risk assessment"[Title/Abstract] OR screening[Title/Abstract]) AND (intimate partner violence[Title/Abstract] OR domestic violence[Title/Abstract]) - 0 results premarital[Title/Abstract] AND (education[Title/Abstract] OR counseling[Title/Abstract]) AND (outcome[Title/Abstract] OR effectiveness[Title/Abstract]) - scanned, no new records selected beyond other angle files already in repo "conflict of interest"[Title/Abstract] AND ("child protection"[Title/Abstract] OR safeguarding[Title/Abstract] OR "mandatory reporting"[Title/Abstract]) "structured professional judgment"[Title/Abstract] AND ("intimate partner violence"[Title/Abstract] OR "domestic violence"[Title/Abstract]) "independent donor advocate"[Title/Abstract] OR "independent living donor advocate"[Title/Abstract]
couple-work-safety-coercive-control	couples therapy intimate partner violence meta-analysis conjoint treatment intimate partner violence exclusion criteria coercive control assessment measurement instrument domestic violence screening couples counseling safety intimate partner violence premarital counseling conjoint couples treatment domestic violence when appropriate coercive control intimate partner violence measurement scale situational couple violence versus coercive controlling violence typology couples therapy contraindicated domestic violence safety concerns intimate partner violence screening tool clinical practice batterer intervention program couples counseling outcomes
decision-autonomy-measures	reproductive autonomy scale validation reproductive coercion scale McCauley OR Miller decisional autonomy marriage women decision-making power measure

Table 1 continued

Search angle	Query string
	informed choice measure family planning validation coercive control recognition instrument scale help-seeking intention behavior intimate partner violence measure psychometric validation autonomy scale Asia OR Muslim marital decision making power scale psychometric "reproductive coercion" AND "scale" Upadhyay "reproductive autonomy" McCauley "reproductive coercion" "decision control" AND marriage "women's autonomy" AND Pakistan OR Bangladesh OR Indonesia OR Malaysia "coercive control" AND "help-seeking" informed choice contraception scale psychometric
digital-self-assessment- ipv	I-DECIDE safety decision aid intimate partner violence myPlan safety decision aid intimate partner violence online safety decision aid domestic violence app randomized anonymous self-screening intimate partner violence acceptability technology based intervention intimate partner violence low and middle income countries web-based intimate partner violence screening self-administered premarital screening intimate partner violence risk assessment mobile app intimate partner violence safety planning online IPV screening Asia acceptability Hegarty I-DECIDE randomised controlled trial safety Glass myPlan intimate partner violence Koziol-McLain iSafe online safety decision aid Ford-Gilboe iCAN online safety intimate partner violence self-assessment tool sexual reproductive health premarital decision aid intervention increase safety behaviour self-efficacy abused women computer based screening intimate partner violence primary care scored self-assessment app mental health youth anonymous
external-culture-citations	<i>not derivable from the logged files (no queries_run log; supplied as an external reading list)</i>
external-dossier-citations	<i>not derivable from the logged files (no queries_run log; supplied as an external reading list)</i>
external-draft-citations	<i>not derivable from the logged files (no queries_run log; supplied as an external reading list)</i>
external-multicultural-clinical-safety	<i>not derivable from the logged files (no queries_run log; supplied as an external reading list)</i>
external-thailand-deepsouth-citations	<i>not derivable from the logged files (no queries_run log; supplied as an external reading list)</i>

Table 1 continued

Search angle	Query string
external-wellbeing-citations	<i>not derivable from the logged files (no queries_run log; supplied as an external reading list)</i>
humility-compassion-self-regulation	(intellectual humility[tiab]) AND (relationship[tiab] OR relational[tiab] OR forgiveness[tiab] OR conflict[tiab]) AND (measure[tiab] OR scale[tiab] OR association[tiab]) relational humility[tiab] AND (forgiveness[tiab] OR conflict[tiab] OR marital[tiab] OR couple[tiab]) cultural humility[tiab] AND (health care[tiab] OR healthcare[tiab] OR outcome[tiab] OR patient[tiab]) cultural humility[tiab] AND (health disparities[tiab] OR patient outcomes[tiab] OR systematic review[pt]) cultural humility[tiab] AND (patient outcomes[tiab] OR clinical outcomes[tiab] OR trust[tiab]) AND (intervention[tiab] OR training[tiab]) self-compassion[tiab] AND (meta-analysis[pt] OR meta-analysis[tiab]) AND (mental health[tiab] OR depression[tiab] OR anxiety[tiab]) self-compassion[tiab] AND (relationship satisfaction[tiab] OR relationship functioning[tiab] OR romantic relationship[tiab]) empathy[tiab] AND (intimate partner violence[tiab] OR IPV[tiab]) AND (perpetration[tiab] OR attitude[tiab]) (self-control[tiab] OR self-regulation[tiab] OR conscientiousness[tiab]) AND (intimate partner violence[tiab] OR partner violence perpetration[tiab] OR coercive control[tiab]) AND (meta-analysis[pt] OR meta-analysis[tiab]) (self-control[tiab] OR low self-control[tiab]) AND (intimate partner violence[tiab] OR dating violence[tiab] OR partner violence perpetration[tiab]) AND (meta-analysis[pt] OR meta-analysis[tiab] OR systematic review[pt]) trust[tiab] AND (romantic relationship[tiab] OR marital relationship[tiab] OR intimate relationship[tiab]) AND (relationship satisfaction[tiab] OR relationship quality[tiab]) accountability[tiab] AND (health care ethics[tiab] OR healthcare ethics[tiab] OR patient trust[tiab]) religiosity[tiab] AND (intimate partner violence[tiab] OR domestic violence[tiab]) AND (systematic review[pt] OR systematic review[tiab]) Islamic psychology[tiab] AND intervention[tiab] AND (randomized controlled trial[pt] OR trial[tiab] OR review[pt]) (humility[tiab] OR values-based[tiab]) AND (couple[tiab] OR marital[tiab] OR premarital[tiab]) AND (intervention[tiab] OR program[tiab] OR education[tiab]) premarital[tiab] AND (education[tiab] OR intervention[tiab] OR program[tiab]) AND (randomized[tiab] OR outcome[tiab] OR effectiveness[tiab]) values-based[tiab] AND (couple[tiab] OR premarital[tiab] OR marriage preparation[tiab])
ipv-screening-response	intimate partner violence screening USPSTF recommendation intimate partner violence screening WHO guideline clinical handbook

Table 1 continued

Search angle	Query string
	intimate partner violence screening Cochrane systematic review effectiveness intimate partner violence referral warm referral health care domestic violence screening harms unintended consequences LIVES first-line support intimate partner violence World Health Organization intimate partner violence guidelines
islamic-medical-ethics-tier-one	Q1: Islamic bioethics[tiab] AND (principles[tiab] OR framework[tiab]) AND (medical decision[tiab] OR clinical decision[tiab]) Q2: Islam[tiab] AND informed consent[tiab] AND (autonomy[tiab] OR family[tiab]) Q3: Muslim[tiab] AND religiosity[tiab] AND (mental health[tiab] OR coping[tiab]) AND systematic review[tiab] [0 hits] Q3b: (spirituality[tiab] OR religiosity[tiab]) AND Muslim[tiab] AND (systematic review[tiab] OR meta-analysis[tiab]) Q4: Islamic[tiab] AND (psychotherapy[tiab] OR intervention[tiab]) AND (randomized[tiab] OR meta-analysis[tiab]) Q5: Islamic[tiab] AND (domestic violence[tiab] OR intimate partner violence[tiab] OR marital[tiab]) AND review[tiab] Q6: Muslim physician[tiab] AND (professionalism[tiab] OR ethics[tiab] OR trust[tiab]) Q7: (FIMA[tiab] OR IMANA[tiab] OR "Islamic medical"[tiab]) AND guideline[tiab] [0 hits] Q7b: Islamic[tiab] AND (medical ethics[tiab] OR professional ethics[tiab]) AND physician[tiab] Q8: maslaha[tiab] OR (Islamic[tiab] AND "harm avoidance"[tiab]) Q9: shared decision making[tiab] AND Muslim[tiab] Q10: Islamic bioethics[tiab] AND reproductive[tiab] Q11: premarital[tiab] AND (Muslim[tiab] OR Islamic[tiab]) Q12: Islam[tiab] AND "family involvement"[tiab] AND decision making[tiab] Q13: Islamic[tiab] AND "BMC Medical Ethics"[Journal] AND (systematic review[tiab] OR review[Publication Type]) Q14: Islam[tiab] AND "Journal of Medical Ethics"[Journal] Q15: Islamic[tiab] AND "The American Journal of Bioethics"[Journal] [0 hits]
manipulation-literacy-inoculation	psychological inoculation misinformation manipulation techniques randomized prebunking manipulation techniques video inoculation coercive control recognition scale measure young people gaslighting recognition emotional abuse education dating violence prevention program recognition attitude school-based randomized Safe Dates program dating violence prevention systematic review dating violence prevention adolescents awareness abuse labelling help-seeking intimate partner violence health literacy access understand appraise apply relationship emotional abuse recognition Asian Muslim women intimate partner violence recognition Thailand

Table 1 continued

Search angle	Query string
	self-check screening tool intimate partner violence anonymous Roozenbeek van der Linden fake news game inoculation health literacy conceptual model access understand appraise apply Sorensen warning signs recognition dating violence education program adoles cents labeling abuse help-seeking intimate partner violence qualitative coercive control awareness scale validation bystander intervention program dating violence recognition training
muslim-thailand- southeast-asia	(intimate partner violence[MeSH] OR IPV) AND (Muslim OR Islam*) AND (Thailand OR Malaysia OR Indonesia OR "Southeast Asia") (forced marriage OR fraudulent marriage OR early marriage OR child marriage) AND Muslim (premarital OR pre-marital) AND (education OR course OR counseling OR counselling) AND (Islam* OR Muslim) help-seeking AND intimate partner violence AND Muslim One Stop Crisis Center Thailand religious leader AND domestic violence AND response unregistered marriage OR religious marriage AND Southeast Asia child marriage AND (Thailand OR Malaysia OR Indonesia) kursus pra nikah OR premarital course Malaysia OR Indonesia Rohingya AND marriage polygamy AND Southeast Asia domestic violence Muslim Malaysia faith based intervention intimate partner violence marriage counseling program evaluation Muslim couples
peer-online-support- digital-safe-space	("intimate partner violence"[tiab]) AND (online[tiab] OR internet[tiab] OR digital[tiab] OR "social media"[tiab] OR forum[tiab]) AND (support[tiab] OR peer[tiab]) ("technology-facilitated abuse"[tiab] OR "digital abuse"[tiab] OR "online surveillance"[tiab] OR cyberstalking[tiab]) AND (partner[tiab] OR relationship[tiab]) AND (systematic[tiab] OR review[tiab] OR abuse[tiab]) ("peer support"[tiab]) AND (online[tiab] OR digital[tiab] OR internet[tiab]) AND (adolescent[tiab] OR "young people"[tiab] OR youth[tiab]) AND ("mental health"[tiab]) AND (systematic review[tiab]) (anonymity[tiab] OR anonymous[tiab]) AND (disclosure[tiab] OR "help-seeking"[tiab] OR "help seeking"[tiab]) AND (online[tiab] OR internet[tiab]) (moderation[tiab] OR moderator[tiab] OR moderators[tiab]) AND ("online support group"[tiab] OR "online community"[tiab] OR "online communities"[tiab] OR "online forum"[tiab] OR "online forums"[tiab]) AND (safety[tiab] OR harm[tiab] OR risk[tiab]) (adolescent*[tiab] OR "young adult"[tiab] OR "young people"[tiab]) AND (Thailand[tiab] OR Asia[tiab] OR "low- and middle-income"[tiab]) AND ("online community"[tiab] OR "peer support"[tiab] OR "digital health"[tiab]) (Muslim[tiab] OR Islamic[tiab]) AND (online[tiab] OR digital[tiab]) AND (health[tiab] OR community[tiab])

Table 1 continued

Search angle	Query string
premarital-education-outcomes	<pre> (premarital OR "marriage preparation" OR "marriage and relationship education" OR "marital education") AND (counseling OR education OR program) AND (outcome OR effectiveness) "systematic review"[pt] AND (premarital OR "marriage preparation" OR "premarital counseling") AND (intimate partner violence OR coercion OR domestic violence) [0 results, reformulated] systematic review[Title/Abstract] AND (premarital[Title/Abstract] OR marriage preparation[Title/Abstract]) AND (violence[Title/Abstract] OR coercion[Title/Abstract]) "Marriage"[Mesh] AND "Counseling"[Mesh] AND ("Intimate Partner Violence"[Mesh] OR "Domestic Violence"[Mesh]) Hawkins AND "marriage education" AND meta-analysis [0 results, reformulated] marriage education[Title/Abstract] AND meta-analysis[Title/Abstract] (faith-based OR religious OR church OR mosque OR clergy) AND ("premarital counseling" OR "marriage preparation" OR "marriage education") AND outcomes premarital counseling AND help seeking premarital education randomized controlled trial "Premarital Examinations"[Mesh] OR "premarital screening" AND consanguinity AND counseling [genetics angle, used only for pool scan, not selected] </pre>
primary-prevention-frameworks	<pre> RESPECT+women+intimate+partner+violence+prevention+WHO intimate+partner+violence+prevention+programme+theory+logic+model SASA!+community+mobilization+intimate+partner+violence global+burden+intimate+partner+violence+prevalence+WHO+2021 faith+based+intervention+intimate+partner+violence+prevention empowerment+relationship+skills+program+intimate+partner+violence RESPECT+framework+preventing+violence+against+women+implementation global+estimates+prevalence+violence+against+women+WHO+Sardinha global+burden+intimate+partner+violence+2021+Global+Burden+of+Disease Indashyikirwa+couples+intervention+intimate+partner+violence+Rwanda Global+Burden+of+Disease+2023+intimate+partner+violence+women GBD+2021+intimate+partner+violence+disease+burden+study </pre>

2.3 Selection of sources of evidence

A single reviewer ran each search pass, read the abstract of each candidate record, and drafted a finding and a relevance annotation. An independent verifier pass (AI-assisted, human-owned) re-fetched every PubMed record by PMID via NCBI E-utilities (no web fetch) and checked title, year, the finding against the abstract, and whether the relevance annotation overstated the abstract; WHO/USPSTF/NICE/Cochrane items not indexed in PubMed were checked by title or identifier search only, not against the live document, and this is flagged per record. Each record received one verdict: KEEP (admitted as drafted), FIX (admitted with the verifier's

correction, shown verbatim in the record), DROP (not admitted), or CONTEXT-ONLY (not PubMed-indexed, retained only as context, no evidential weight). This is single-reviewer screening with an independent adversarial verification pass, not independent duplicate screening by two reviewers as a full systematic review would require; this is the process’s central limitation and is stated as such throughout, not only here.

2.4 Data charting items

For every admitted record, the library charts: identifier (PMID or, for a guidance document, its own identifier), category, first author, title, year, journal, design, evidence level, population or setting, finding, a relevance annotation linking the record to the T-PHE programme theory, the verifier’s verdict and note, and (where applicable) the other categories the same record was also placed in. The knowledge graph (`library/kg/graph.json`) additionally links each record to zero or more of the seven propositions (P1–P7), by a relation of `supports`, `complicates`, `relates_to` (relevant only), or `contradicts`; to the safeguards and outcome domains it measures or discusses; and to its category.

2.5 Synthesis approach

No meta-analysis was performed and none is appropriate: the library spans burden estimates, guidance documents, systematic reviews, single RCTs, qualitative studies, psychometric validation studies and narrative reviews across 18 categories with no common outcome metric. Synthesis is narrative, organised two ways: by category (Results §3.2) and by proposition P1–P7 (Results §3.3, one table per proposition, listing every linked record with its design, population, finding as charted, and its direction relative to the proposition). No risk-of-bias tool was applied at the individual-study level; a future full systematic review of any one proposition would apply ROBIS or AMSTAR-2 to included systematic reviews and Cochrane RoB 2 to included randomised trials, neither of which was in scope for a scoping-level evidence map.

2.6 Limitations of the process

(1) Single-reviewer screening throughout, with an AI-assisted independent verifier pass as the only check, not independent duplicate human screening. (2) No protocol was registered before the search began; no PROSPERO record exists. (3) Search scope was fixed to PubMed-indexed literature and five named global-guidance bodies (WHO, USPSTF, NICE, Cochrane, MRC); grey literature, Thai national databases such as ThaiJO/TCI, and non-English sources were excluded by design and are logged only where a search pass happened to attempt and fail to resolve one (`library/library.json context_sources`). (4) No risk-of-bias assessment was applied at the individual-study level. (5) No meta-analysis was performed or attempted. (6) The 18 category searches were run across different sessions with different query strategies (12 with a logged PubMed query set, 6 as supplied reading lists), so search sensitivity is not uniform across categories; this is a stated limitation, not something the counts in §3.1 attempt to correct for. (7) design and evidence-level fields were not populated for the six externally-sourced categories (07–12) in the logged data, so the design/evidence-level distribution in Table 4 undercounts those 96 entries as blank rather than reconstructing a design label that was never charted. (8) **Data-quality issue in the source library, resolved downstream, not upstream.** Two genuinely different admitted documents – a WHO 2013 clinical/policy guideline and a distinct WHO 2014 clinical handbook, both charted in `library/library.json` category 03 – were left with a blank `identifier` field, so both would naively collide onto the same identifier-less key and only one (the 2014 handbook) would ever appear in the rendered tables, silently dropping the 2013 guideline’s own finding from every proposition table it is linked to. `library.json` itself was not edited (per this repository’s separation between the public `library/` and this private

manuscript’s build scripts); instead, both `build_prisma.py` and `generate_tex_fragments.py` resolve the collision with a deterministic fallback key (PMID if present, otherwise a slug of `first_author+year+title`), so the two documents are counted and tabulated as the two distinct records they are. This is flagged here as a data-quality issue of the source library, to be corrected upstream (assigning the 2013 guideline its own non-blank identifier in `library.json`) in a future revision of that repository. A related point not to be conflated with the identifier fix: `kg/graph.json` records only one `relates_to` edge from the collided source id to each of P2 and P5, so the two documents’ appearance as separate rows in Tables 6 and 9 rests on applying that single knowledge-graph edge to both resolved documents, not on two independently coded relations – the proposition link itself is exactly as recorded in the knowledge graph, no more and no less.

3 Results

3.1 Selection of sources of evidence (PRISMA flow)

Figure 1 and Table 2 report the PRISMA-ScR flow, computed directly from the logged files by `build_prisma.py`. Records identified through search (358, sum of hits across 18 angle files, 160 logged PubMed queries) de-duplicate to 277 unique PMIDs among the PMID-bearing candidates (285 of the 358 candidates carried a PMID at the search stage; the remaining 73 were guidance documents or non-PMID items in the six externally-sourced angle files, and are not part of the PMID de-duplication count). All 358 candidate records were passed to the independent verifier: 250 KEEP, 97 FIX, 2 DROP, 9 CONTEXT-ONLY. The library admits 347 category-level entries, corresponding to 330 unique records (313 unique PMIDs plus 17 admitted guidance documents without a PMID); the gap between 347 and 330 is records placed in more than one category (`also_in_category`), each counted once per category it appears in. Of the 18 non-PMID rows, one pair (a WHO 2013 clinical/policy guideline and a distinct WHO 2014 clinical handbook, both charted with a blank `identifier` field in `library.json`) would naively collide onto the same identifier-less key; both scripts resolve this with a deterministic fallback key (PMID if present, otherwise a slug of `first_author+year+title`) so the two documents are counted, and tabulated, separately – see Limitations.

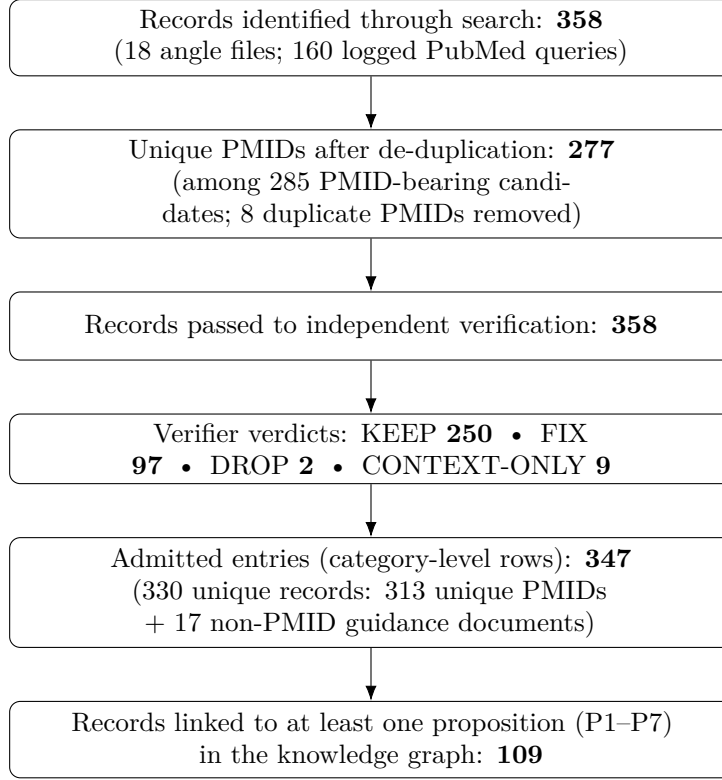


Figure 1: PRISMA-ScR flow. Every number is computed by `build_prisma.py` from `library/data/search-*.json`, `library/data/verify-*.json`, `library/library.json` and `library/kg/graph.json`.

Table 2: PRISMA-ScR flow counts, computed by `build_prisma.py` directly from `library/data/search-*.json`, `library/data/verify-*.json` and `library/library.json`.

Stage	n
Records identified through structured PubMed E-utilities searches and logged reading-list passes (sum of hits across 18 search-*.json angle files)	358
Total logged PubMed E-utilities queries run (queries_run arrays)	160
Duplicate PMIDs removed across search files	8
Unique PMIDs identified by search	277
Records passed to the independent verifier	358
Verifier verdict: KEEP	250
Verifier verdict: FIX (correction applied, then admitted)	97
Verifier verdict: DROP (not admitted)	2
Verifier verdict: CONTEXT-ONLY (not PubMed-indexed; no evidential weight)	9
Admitted entries, category-level rows (a record placed in more than one category counts once per category)	347
Admitted unique records (PMID, or identifier for a non-PMID guidance document)	330
Admitted unique PMIDs	313
Admitted entries without a PMID (WHO/NICE/Cochrane/JAKIM/Malaysia-government guidance)	18
Not admitted (DROP, library.json not _admitted list)	2
Context-only sources logged (not PubMed-indexed, carry no evidential weight)	9

3.2 Characteristics of included records

Table 3 reports admitted entries per category, cross-checked between the count declared in `library.json` and a direct recount over the entries themselves (the two agree for every category). By design/evidence-level keyword count over the free-text fields as logged (Table 4), 41 admitted entries (11.8% of 347) read as a systematic review, scoping review, meta-analysis, or Cochrane review by keyword match; 12 (3.5%) read as a guideline or recommendation statement, matched only by the two keywords the row label names (“guideline” and “recommendation statement”; no broader term such as “guidance” is matched, so a record is not counted here merely for mentioning guidance in passing); and 59 (17.0%) read as a randomised or cluster-randomised controlled trial. These are keyword counts over free text, not a closed design taxonomy applied by a second coder, and are reported as such. The design and evidence-level fields were not populated for the 96 entries admitted from the six externally-sourced categories (07–12); this is a property of the logged data, not a search-strategy failure, and is reported as “not derivable from the logged files” rather than reconstructed. Admitted entries span publication years 1991–2026, with 70 of 347 entries (20.2%) dated 2024 or 2025 and 36 dated 2026 (year-bucketing rule, applied identically by `build_prisma.py`: a `year` value counts toward a numeric year only if it is an integer or a clean 4-digit string; every other value – blank, or an annotated/range string such as “2021 (base report); 2023/2024 update per draft claim” – is bucketed under “year not parsed” and shown as its own row rather than folded into any numeric year; 7 of 347 entries fall in that row. The full year distribution is in the accompanying `prisma_counts.md`).

Table 3: Admitted entries per category (`library.json` declared count vs. direct recount over entries; the two agree in every row).

#	Category	Admitted
01	Burden of intimate-partner violence and primary-prevention frameworks	20
02	Premarital and relationship education: measured outcomes	20
03	Screening, first-line response and referral	21
04	Couple-based work and coercive control	20
05	Decision autonomy, reproductive coercion and measurement instruments	18
06	Muslim populations, Thailand and Southeast Asia	23
07	Multicultural clinical safety and language access	19
08	Preconception medicine, premarital screening and adjacent guidance	17
09	Global guidance, frameworks and methods	29
10	Thailand and the southern border provinces	15
11	Premarital knowledge and well-being pathways	7
12	Culture as a health determinant and intercultural coexistence	9
13	Digital self-assessment and safety decision aids for IPV	20
14	Peer online support, moderated communities and digital safe spaces	20
15	Assessor independence, risk assessment and check-in models	24
16	Recognition of coercive control and psychological manipulation; inoculation	20
17	Humility, compassion, self-regulation and responsibility	25
18	Islamic medical ethics and spirituality in Muslim health care (tier-one only)	20
Total (category-level rows)		347

Table 4: Design/evidence-level keyword shares over the free-text `design` and `evidence_level` fields as logged (not a closed taxonomy; a record can match more than one group, or none). The “Guideline / recommendation statement” row matches only the two keywords its label names – the bare substrings “guideline” and “recommendation statement” – and deliberately excludes the broader term “guidance,” so a record is not counted here merely for mentioning guidance in passing. The “RCT / cluster-RCT” row matches the substrings “randomised controlled trial,” “randomized controlled trial,” “rct,” “cluster randomi,” “randomised trial” and “randomized trial” (case-insensitive).

Keyword group	n entries	Share of 347 admitted entries
Systematic review / meta-analysis / scoping review / Cochrane	41	11.8%
Guideline / recommendation statement	12	3.5%
Randomised or cluster-randomised controlled trial	59	17.0%

3.3 Evidence by proposition (P1–P7)

Tables 5–11 chart every record the knowledge graph links to each proposition: record and design and population as charted in the library, the finding exactly as charted there (copied verbatim from the record’s `finding` field, or its `verifier_note` where `finding` was blank), and the direction of that record relative to the proposition (`supports`, `complicates`, or `relates_to` shown as “Relevant only”; no record in the graph is tagged `contradicts` any proposition, an absence reported in §4.1, not filled in with a record that was not tagged that way). Record counts per proposition are uneven by construction of the search categories, not by omission: P1 (decision control, the paper’s central mechanism) draws on burden, instrument-validation and community-mobilisation literatures spanning several categories (44 linked records); P6 (conventional success can coexist with T-PHE failure) and P7 (repairable records) are the two propositions argued mainly by analogy from a small number of directly relevant records (6 each).

P1 — A governance layer improves decision control beyond content-only education

Table 5: P1: A governance layer improves decision control beyond content-only education. Record, design, population/setting, finding as charted (verbatim from the library record), and direction relative to the proposition. n=44 linked records (kg/graph.json edges).

Record	Design	Population/ setting	Finding (as charted)	Direction
Abramsky T 2014 [14]	Pair-matched cluster randomized controlled trial (8 comm unities)	Community members aged 18-49, Kampala, Uganda; baseline n=1,583, follow-up n=2,532	SASA! was associated with significantly lower social acceptance of IPV among women (aRR 0.54, 95% CI 0.38-0.79) and 52% lower past-year physical IPV among women (0.48, 95% CI 0.16-1.39, crossing 1), plus higher likelihood of women receiving supportive community responses to disclosed violence.	Supports
Alghamdi M 2022 [83]	Qualitative dominant design with su pplemental qu antitative data (n=8)	Canadian Muslim immigrant women, 6 countries of origin	Pre-migration adverse childhood experiences and family history of IPV, combined with post-migration language/social-isolation barriers and internalized patriarchal values, shaped help-seeking and the eventual decision to leave abusive relationships among 8 Muslim immigrant women.	Supports

Record	Design	Population/ setting	Finding (as charted)	Direction
Blackwood E 2007 [57]	Historical/ discourse review	Indonesia, state/ religious/ activist discourse, 1980-2000s	Traces how Indonesian state and Islamic discourse shifted from linking normative gender to heterosexual marriage toward more direct legal regulation of sexuality via criminal-code proposals, illustrating how marriage has functioned as a governance mechanism in national policy discourse.	Supports
Boyce SC 2023 [93]	Item response modelling (Partial Credit Model) on population- based dyadic survey	559 married adolescent-girl- husband dyads (ages 13-18 wives), rural Niger, 2019	A 5-item, 2-dimensional social-norms scale (challenging husband authority; general IPV acceptability) showed reliability and validity; higher 'challenging husband authority' scores were statistically associated with husband perpetration of IPV.	Supports
Böttcher Bettina 2025 [104]	Cross-cultural validation study	Austrian student population (including men)	The RAS 3-factor structure was supported; decision-making and freedom-from-coercion subscales had satisfactory internal consisten cy, while the communication subscale showed marginally lower reliability; use in men and gender-diverse individuals was noted as constrained, requiring adaptation.	Supports
Couture- Carron A 2020 [76]	Exploratory q ualitative study	South Asian Muslim comm unities (diaspora)	Cultural norms constructing dating as shameful were found to intensify fear of parental/community reaction and strengthen attachment to abusive partners, with direct implications for disclosure, help-seeking, and the ability to end abusive relationships.	Supports
GBD 2023 IPV and SVAC Collabo rators 2026 [1]	GBD modelling study (systematic analysis, spati otemporal Gaussian process regression + burden-of- proof risk met hodology)	204 countries and territories, females aged 15+ (IPV) and males/females (sexual violence against children), 1990-2023	In 2023, an estimated 608 million (95% UI 518-724) females aged 15+ had ever been exposed to IPV, contributing 18.5 million (8.74-30.0) DALYs; IPV ranked as the 4th leading risk factor for DALYs among females aged 15-49. Anxiety disorders and major depressive disorder were the leading IPV-attributed health outcomes (5.43 million and 3.96 million DALYs respectivel y).	Supports
Gausman J 2022 [90]	Qualitative co mparative study, 8 focus group discussi ons (4 per country)	Syrian refugees (Jordan) and Rohingya refugees (Bangladesh), fathers/ mothers/adole scents	In both displaced Muslim-majority populati ons, participants described child marriage norms reinforced across generations, with girls themselves describing having little agency in marriage decisions, alongside contextual resistance-to-change dynamics.	Supports

Record	Design	Population/ setting	Finding (as charted)	Direction
Gausman Jewel 2023 [95]	Psychometric validation on cross- sectional multi-country survey data	Women in Argentina (n=890), Ghana (n=1,114), and India (n=1,130)	[CORRECTION: key_finding is accurate: MADM adapted to FP-ADM, validated in Argentina (n=890), Ghana (n=1,114), India (n=1,130), two-factor solution ('Bidirectional Exchange of Information', 'Empowered Choice'), good internal consistency. PROBLEM: relevance_to_tph claims India's inclusion is 'relevant to Thailand's regional/cultural proximity' – India is South Asia, Thailand is Southeast Asia; this is a geographic overreach, not a supported cultural-proximity claim (no shared language family, religion-majority, or documented cultural closeness argued in the source). FIX: remove or soften the 'regional/cultural proximity to Thailand' framing; state only that a South Asian (not Southeast Asian, not Muslim-specific) LMIC subsample exists, without implying meaningful proximity to the Thai Muslim/i ntercultural target population.] Adapted the Mothers Autonomy in Decision-Making (MADM) scale to contraceptive counselling contexts, producing the FP-ADM scale, and validated its psychometric properties across three culturally distinct LMIC settings including a South Asian one (India).	Supports
Han X 2024 [97]	Cross- sectional, random- effects Poisson regression (n=201)	Married Salar (Turkic- Muslim minority) women, rural China	Among 201 married Salar Muslim women, 65.6% reported past-year child neglect, 30.8% reported past-year IPV, and 37.6% had experienced child marriage; child marriage and IPV victimization were associated with downstream child-neglect risk.	Supports
Hawkins AJ 2012 [16]	Meta-analysis (148 evaluation reports)	Mixed university/ laboratory and religious- setting MRE programs, largely US	Moderate-dosage programs (9-20 contact hours) had stronger effects than low-dosage (1-8 hours); a communication-skills emphasis strengthened communication outcomes but not satisfaction; no evidence that formal institutionalization (manuals, trainer certification) improved effects, and no significant difference between religious-setting and university/laboratory delivery.	Supports

Record	Design	Population/ setting	Finding (as charted)	Direction
Hazar Seda 2024 [96]	Cross-cultural adaptation and psychome- tric validation (online survey, EFA/CFA)	308 married women of repr- oductive age (15-49), Türkiye, recruited via WhatsApp/ Instagram	[CORRECTION: Abstract confirms 14 items/3 subscales and four alpha values (0.66 total, then 0.64, 0.89, 0.92) with test-retest high correlation. PROBLEM: the abstract does NOT state which named subscale (decision-making / freedom-from-coercion / communication) c- orresponds to the 0.64 alpha value – it lists 'reproductive autonomy scale (RAS) total and subscale ... 0.66, 0.64, 0.89, and 0.92, re- spectively' without labeling order. The record's specific attribution 'the decision-making subscale showing the lowest reliability' is therefore NOT confirmed by the abstract alone. FIX: either verify against the full text which subscale=0.64 and cite that specifically, or soften to 'one subscale showed markedly lower reliability (alpha=0.64); the abstract does not specify which subscale this is' – do not name 'decision-making' without full-text confirmat- ion, since this is exactly the kind of psycho- metric-number precision this review is asked to be strict on.] The Reproductive Autonomy Scale retained its 3-subscale (14-item) structure in Turkish; subscale Cronbach's alpha ranged from 0.64-0.92 (total scale 0.66), with the decision-making subscale showing the lowest reliability among subscales tested.	Supports

Record	Design	Population/ setting	Finding (as charted)	Direction
Hill AL 2019 [74]	Secondary cross-sectional analysis of baseline survey data	550 sexually active female high school students, USA (school health center attendees, ages 14-19)	[CORRECTION: Prevalence numbers are correct (12% recent RC, 17% ARA) among 550 sexually active female students. PROBLEM: key_finding claims 'both were associated with differences in care-seeking and sexual health behaviors' – but the abstract EXPLICITLY states the opposite for reproductive coercion alone: 'There were no observed significant differences in care-seeking behaviors among those with recent reproductive coercion compared with those without.' It is adolescent relationship abuse (ARA), not reproductive coercion, that was associated with a care-seeking difference (increased odds of STI testing/treatment, aOR 2.08); the combined ARA+RC-exposed group showed differences in sexual-partner/contraception behaviors, not general 'care-seeking'. FIX: rewrite key_finding to state that RC alone showed NO significant care-seeking difference, ARA alone was associated with increased STI-testing care-seeking (aOR 2.08), and the ARA+RC combined group showed elevated odds of specific risk behaviors (older partner, more partners, hormonal-only contraception) – do not state both constructs were 'associated with differences in care-seeking'.] 12% reported recent reproductive coercion and 17% reported adolescent relationship abuse; both were associated with differences in care-seeking and sexual health behaviors, measured via validated RC and relationship-abuse instruments.	Supports
Islam MK 2021 [84]	Mixed methods: cross-sectional survey (n=96) + qualitative interviews (n=18) + provider interviews (n=9)	Rohingya adolescent girls (age 10-19) with experience of child marriage, Cox's Bazar refugee camp, Bangladesh	Mean age at first marriage was 15.7 years; over 80% had given birth or were pregnant within two years of the survey; key drivers of child marriage included perceived physical/mental maturity norms, social norms, insecurity, and family honour.	Supports
Islam MK 2021 [5]	Cross-sectional survey, multivariable logistic regression (n=486)	Rohingya refugee women, Cox's Bazar settlement, Bangladesh	61.32% of surveyed women had experienced child marriage; child marriage was associated with significantly higher odds of justifying husband-perpetrated physical abuse (AOR=2.71, 95%CI 1.78-4.11) and of experiencing IPV in the past 12 months (AOR=1.72, 95%CI 1.13-2.61).	Supports

Record	Design	Population/ setting	Finding (as charted)	Direction
Jacobowski N 2008 [45]	Documentar y/archival analysis and expert- informant interviews	Married women, Indonesia (Muslim- majority context)	Traditional practices including polygamy, early marriage and contract marriage (mut'a) were identified as enhancing married Indonesian women's likelihood of HIV acquisition within marriage, challenging the assumption that marital status is protective.	Supports
Kurnaz Kaan 2026 [105]	Cross- sectional desc riptive-correla tional survey with path analysis	480 married women aged 18-49, Türkiye	[CORRECTION: Title/year/design/population match exactly. PROBLEM: key_finding claims 'reproducti ve coercion and traditional gender-role attitudes were linked to fertility desire pathways' via path analysis, but the abstract's Results section only reports a path-analysis link between the Reproductive Coercion Scale and Fertility Desire Scale ($r=0.172$, $p<0.001$; $\beta=0.205$, $p=0.008$) and does NOT report a statistically establis hed gender-role-attitude -> fertility-desire path – the Gender Role Attitude Scale is only mentioned as a measure administered (mean score 121.92 ± 15.14), not as part of a reported path result. FIX: rewrite key_finding to state only the RCS-FDS path-analysis association (weak positive, $r=0.172/\beta=0.205$) and drop or explicitly qualify the gender-role-attitudes-linked-to- fertility-desire claim as not shown in the abstract.] Used the Reproductive Coercion Scale, Fertility Desire Scale, and Gender Roles Attitude Scale together; reproductive coercion and traditional gender-role attitudes were linked to fertility desire pathways in married women (path analysis).	Supports
Kuswanto H 2024 [3]	Cross- sectional, binary logistic regression (nationally re presentative, $n=9,333$)	Females aged 15-20, Indonesia (2017 Indonesia Dem ographic and Health Survey)	National prevalence of female child marriage in Indonesia was 12.53%; health insurance status and sex of household head were not significant predictors, while other socioeconomic/education factors were associated with child marriage risk.	Supports
McCollum 2008 [21]	narrative review of outcome research and clinical practice	IPV-affected couples in conjoint treatment settings, US clinical literature	Concludes conjoint IPV treatment can be effective and safe only when embedded in a broader community response, with careful screening of couples for inclusion, modification of standard conjoint methods to promote safety, and ongoing safety assessment with contingency plans for escalation; a 'one-size-fits-all' approach is explicitly rejected given IPV typologies.	Supports

Record	Design	Population/ setting	Finding (as charted)	Direction
Mieth K 2025 [100]	Qualitative study (49 in-depth interviews, 16 focus group discussions)	Forcibly Displaced Myanmar Nationals (Rohingya) aged 15-24, Cox's Bazar, Bangladesh (post-October 2016 arrivals)	Participants reported perceived increases in child marriage rates post-displacement, attributed to protection fears, socioeconomic need, lost education/employment opportunity, and perceived loosening of legal-age enforcement in the camp context.	Supports
Riches Eleanor 2023 [91]	Cross-sectional online survey validation (classical test theory + CFA)	826 women of reproductive age, United Kingdom	The RAS was highly acceptable (>97.7% response rate) with good internal consistency (Cronbach's alpha 0.75) and fair-good test-retest reliability (ICC 0.67); construct validity was supported via confirmatory factor analysis.	Supports
Rogers MM 2022 [92]	Qualitative interview study	Family medicine providers/staff at two US academic outpatient centers	[CORRECTION: first_author is wrong: search lists 'Rogers MM' but PubMed indexes the first author as Singh. Numbers confirmed exactly: 'One in five men report lifetime intimate partner violence (IPV) perpetration...Two in three male IPV perpetrators seek routine health services...family medicine physicians feel unprepared to screen male patients for IPV due to lack of knowledge and training.' FIX: change first_author to Singh.] Notes that one in five men report lifetime IPV perpetration and two in three male perpetrators seek routine health services, yet family medicine physicians feel unprepared to screen male patients for IPV perpetration due to lack of knowledge/training, despite validated screening tools existing.	Supports
Rumble L 2018 [4]	Cross-sectional secondary analysis, multivariate regression (nationally representative, n=6,578)	Women aged 20-24, Indonesia (2012 Indonesian Demographic and Health Survey / Adolescent Reproductive Health Survey)	In a nationally representative Indonesian sample, approximately 17% of women were married before age 18 and 6% before age 16; the paper notes research on child marriage in Southeast Asia specifically was scarce prior to this nationally representative multivariate analysis.	Supports

Record	Design	Population/ setting	Finding (as charted)	Direction
Sardinha L 2022 [2]	Systematic review + Bayesian multilevel modelling of prevalence data	366 eligible studies, 2 million women, 161 countries/ areas, ever-partnered women aged 15-49	[CORRECTION: 27% (UI 23-31%) lifetime and 13% (10-16%) past-year figures and the 24% (21-28%) for age 15-19 figure are verbatim-correct. But the record states '26% aged 20-24' while the fetched abstract literally reads '26% (23-30%) of women aged 19-24 years' (likely an upstream GBD/abstract typo, but the record must match the source text as fetched). FIX: change '20-24' to '19-24' to match the abstract exactly, or flag the age-band as ambiguous in a footnote.] Globally, 27% (UI 23-31%) of ever-partnered women aged 15-49 have experienced physical or sexual, or both, IPV in their lifetime, with 13% (10-16%) in the past year; violence starts early, affecting 24% (UI 21-28%) of women aged 15-19 and 26% (23-30%) aged 20-24.	Supports
Sharifnia A 2025 [98]	Meta-ethnogr aphy (33 qual itative studies synthesized), 6 databases searched	Muslim women, global (multi-country qualitative evidence)	Synthesis of 33 qualitative studies identified four major themes - experience/impact of abuse, risk factors, help-seeking, and coping - and found religion functions both as a risk factor for abuse and a barrier to help-seeking, and simultaneously as a coping resource for some women.	Supports
Stern E 2020 [75]	Longitudinal qualitative study	Heterosexual couples partici pating in the I ndashyikirwa curriculum, Rwanda	[CORRECTION: Key_finding says the programme 'was associated with reported shifts...over time' but omits the abstract's explicit caveat that shifts were only 'moderate, but significant,' and that 'these shifts occurred without fully transforming deeply-entrenched beliefs and norms,' which the authors say could constrain longevity of change. Given T-PHE's own epistemic-humility/FAILURE CRITERION framing, this caveat should be added rather than presenting the finding as an unqualifie d positive shift.] The curriculum, designed to build relationship skills and reduce gender-inequitable beliefs/behaviours/norms underpinning IPV, was associated with reported shifts in individual beliefs, couple r elationship dynamics and gender norms among participating couples over time.	Supports

Record	Design	Population/ setting	Finding (as charted)	Direction
Stern E 2021 [81]	Comparative qualitative case study (secondary thematic analysis)	Participants across four IPV-prevention programmes, What Works to Prevent Violence against Women and Girls Programme (Ghana, Rwanda, South Africa, Tajikistan)	Similar cross-context pathways of change were identified: learning and applying relationship skills, participatory challenge of harmful gender norms with group rapport, and integration of economic-empowerment activities to reduce drivers of IPV and build self-confidence.	Supports
Taft 2024 [25]	randomized controlled trial, n=138 couples	US military service members and partners on a military installation	Service members randomized to the couples-based Strength at Home Couples (SAH-C) prevention program showed greater effect-size reductions than a comparison Supportive Prevention condition across physical, sexual, psychological, and specifically coercive-control IPV, plus greater reductions in suicidality; partner-reported effects were in the same direction but less robust.	Supports
Thombs BD 2017 [66]	Comparative methodological review of guideline organizations (includes IPV screening as one of 22 compared recommendations)	Cross-national comparison of screening guideline bodies (Canada, UK, US)	Among 22 questionnaire-based screening recommendations compared across three major guideline bodies (including IPV screening), recommendations diverged substantially, and direct RCT evidence for the effectiveness of the questionnaire-based screening itself (as opposed to downstream treatment) was often absent or indirect.	Supports
Upadhyay UD 2014 [9]	Instrument development and validation (cross-sectional survey, factor analysis)	1,892 women at 13 family planning and 6 abortion facilities, USA	Generated 26 candidate items, retained 14 items across 3 subscales (freedom from coercion, communication, decision-making) via factor analysis; the freedom-from-coercion and communication subscales were inversely associated with unprotected sex in the past 3 months, supporting construct validity.	Supports

Record	Design	Population/ setting	Finding (as charted)	Direction
Ratnaningsih M 2022 [88]	Cross-sectional survey, bivariate/ANOVA/logistic regression (n=1,000 households)	Households, Bone District (South Sulawesi) and Palu/Sigi/Donggala (Central Sulawesi), Indonesia	[CORRECTION: Abstract supports: household financial security (p=0.016), dowry (p<0.001) and legal frameworks (p=0.017) as significant ANOVA-associated factors with child marriage acceptability in Bone/South Sulawesi and Palu-Sigi-Donggala/Central Sulawesi. However, the record's clause 'across two Indonesian provinces where child-marriage rates remain above the national average' is NOT supported for these specific study sites – the abstract's '22 out of 34 provinces... above the national average in 2019' statement is a general national context sentence, not a claim tied to Bone/South Sulawesi or Central Sulawesi by name. FIX: change to something like 'two Indonesian provinces' and drop or separately attribute the 'above national average' clause as general Indonesia-wide context rather than a site-specific fact.] Household financial security, dowry practices and local legal frameworks were significantly associated with community acceptability of child marriage across two Indonesian provinces where child-marriage rates remain above the national average.	Supports
Ghuman SJ 2003 [26]	Comparative study, 15 matched Muslim/non-Muslim community pairs	Married women, India, Malaysia, the Philippines and Thailand	Women's autonomy was not consistently lower in Muslim than non-Muslim communities across the four Asian countries studied (including Malaysia and Thailand), and the association between autonomy measures and child mortality was weak both across and within communities.	Complicates
Hawkins AJ 2008 [6]	Meta-analysis (86 reports, 117 studies, >500 effect sizes)	Mostly US/Western MRE program participants; predominantly White, middle-class samples	Relationship-quality effect sizes for experimental studies ranged d=.30-.36 and communication-skills effects d=.43-.45; moderate-dosage programs outperformed low-dosage; the authors flag that racial/ethnic and economic diversity was too thin to draw reliable conclusions for disadvantaged couples, and that outcomes policymakers care about (relationship stability, aggression) were rarely measured.	Complicates

P1 continued

Record	Design	Population/ setting	Finding (as charted)	Direction
Markman HJ 2013 [7]	Randomized controlled trial (couples via religious o rganizations randomized to PREP vs. naturally occurring premarital services; n=44/66/83)	US couples marrying through religious organ izations, 8-year follow-up on divorce	No overall difference in divorce rates between PREP (clergy-delivered or professio nal-delivered) and usual premarital services; couples with higher premarital negative-com munication observed pre-intervention were MORE likely to divorce if given PREP, while low-negativity couples benefited; a premarital history of physical aggression showed a similar (marginally significant) harmful-moderation pattern.	Complicates
Miller Elizabeth 2016 [43]	Cluster randomized controlled trial (ARCHES)	4,009 females aged 16-29 across 25 family planning clinics (17 clusters), USA	The universal education/counseling interven tion (ARCHES) did NOT significantly reduce reproductive coercion (ARR 1.50, 95% CI 0.95-2.35) or partner violence (ARR 1.07, 95% CI 0.84-1.38) at 1-year follow-up, though it improved secondary outcomes such as recognition and self-efficacy.	Complicates
Phonkacha N 2026 [106]	Cross- sectional national survey, binary probit regression	General population, national survey, Thailand (Muslim identity modeled as a covariate)	Muslim identity, poverty, and residence in southern/central Thailand correlated with higher IPV tolerance for both genders; internet access (as distinct from device ownership) significantly reduced IPV tolerance, suggesting a role for information access in shifting attitudes.	Complicates

Record	Design	Population/ setting	Finding (as charted)	Direction
Bhandari TR 2014 [63]	Scale construction and validation (24-item pool, psychometric testing, cross-validation across two districts)	Women in Rupandehi and Kapilvastu districts, Nepal (South Asian, patriarchal marital-household context)	[CORRECTION: Psychometric numbers in key_finding are accurate: Cronbach's alpha 0.84, test-retest r=0.87, content validity ratio 0.8, good convergent/discriminant validity – all quoted directly from abstract. PROBLEM: relevance_to_tphe/key_finding both assert the scale is 'validated specifically for South Asian household/marital decision contexts' and is a 'household-decision-power-focused autonomy scale', but the abstract never describes the item content – it defines the construct only as generic 'women's autonomy... relevance to developing countries' and its outcome anchor is maternal health care service utilization, not household/marital decision-making specifically. FIX: remove or soften 'household/marital decision contexts' – state that the abstract does not disclose item content, so the household/marital-decision framing is an inference, not an abstract-confirmed fact; also soften 'South Asian' generalization since it was validated only in two districts of one country (Nepal), not South Asia broadly.] Produced a 23-item women's autonomy scale with strong psychometric properties (Cronbach's alpha 0.84, test-retest r=0.87, content validity ratio 0.8), validated specifically for South Asian household/marital decision contexts, showing good convergent and discriminant validity.	Relevant only
Clyde TL 2020 [48]	Narrative review/position paper	General discussion of contemporary premarital-education practice, US-centric	Reviews social trends (individualism/commitment ambivalence, changing marriage attitudes, premarital relationship histories, media environment) and argues current premarital interventions need updated content and structure to remain relevant to engaged couples today; offers four proposals for redesigning premarital interventions.	Relevant only

Record	Design	Population/ setting	Finding (as charted)	Direction
Kyegombe N 2017 [64]	Qualitative dyadic study (in-depth interviews, framework analysis)	Couples in SASA! intervention communities, Kampala, Uganda	[CORRECTION: Key_finding content is supported by the abstract. However, first_author is wrong: the record lists 'Kyegombe N' but PubMed's actual first author for PMID 27682273 is 'Starmann E' (Kyegombe N appears to be a co-author, and is the correct first author on a different SASA! paper, PMID 30613427). FIX the first_author field to Starmann E before citing.] Engagement with SASA! activities and relationship-skills/communication learning was associated with reduced conflict and IPV for many couples and increased trust, respect and joint economic problem-solving, though experiences and degree of change varied widely across couples.	Relevant only
Park J 2026 [99]	Quasi-experimental study with inverse probability of treatment weighting (weighted N=291 women, 228 men)	Couples in a city-wide (Seoul) premarital education program, South Korea, during COVID-19	[CORRECTION: Key_finding matches: 'S-PEP via VC was effective in increasing levels of marital readiness for both women and men and in enhancing relationship confidence and satisfaction for women.' Year check: PubMed's JournalIssue citation year is 2026 (Prevention Science assigned this to a 2026 issue), but the article's actual electronic publication / PubMed indexing date is 2025-01-06 and the study itself was conducted during 'the early COVID-19 period' (i.e. ~2020-2021 data). This is a legitimate ahead-of-print/issue-year artifact, not an error, but FIX: add a one-line caveat in the record noting the 2026 tag is the journal's assigned print-issue year, not the study's data-collection period, so a reader doesn't assume this is a 2026 study of a 2026 phenomenon.] Videoconferencing delivery of the Seoul Premarital Education Program (S-PEP) was associated with increased marital readiness for both men and women, and with increased relationship confidence and satisfaction for women specifically.	Relevant only
Portnoy 2020 [79]	qualitative study — patient interviews (n=10) and provider focus groups (n=29)	US Veterans Health Administration patients and providers	Identifies convergent and divergent patient/provider beliefs about barriers and facilitators to screening specifically for IPV perpetration (not just victimization), an area with a much thinner evidence base than victimization screening.	Relevant only

Record	Design	Population/ setting	Finding (as charted)	Direction
Semahegn A 2017 [67]	Study protocol for a community- based quasi-e xperimenta l/cluster trial	1,269 women planned across intervention, active comparator and control groups, Awi zone, northwes tern Ethiopia	The protocol describes a planned pilot of tr anslating existing evidence and policy into community-level domestic-violence prevention via peer education, community r epresentative training, husband involvement and integration with community health extension programmes; no outcome data are reported (protocol only).	Relevant only
US Preventive Services Task Force 2004 [56]	guideline / re commendatio n statement	US general population, family and intimate partner violence screening in primary care	[CORRECTION: Title, year (2004), journal (Annals of Internal Medicine), first_autho r all match. BUT the PubMed abstract for this record is purely procedural/administrat ive – it describes the process ('In 1996, the USPSTF found insufficient evidence to recommend for or against...a grade C recom mendation, according to 1996 grade definiti ons. The USPSTF now uses an explicit process...') and does NOT itself state the 2004 recommendation's actual grade or conclusion text (no 'I statement' or 'insuffici ent evidence' language about the 2004 finding appears in the abstract retrieved via efetch). The key_finding's specific claim ('USPSTF found the evidence insufficient (I statement) to recommend for or against routine screening instruments...') is historic ally well-established (corroborated indirectly by PMID 23338828's abstract, which refers to updating '2004' with a new B-grade) but is not directly supported by this record's own PubMed abstract text. FIX: either cite a source that states the 2004 grade explicitly, or soften key_finding to note the grade is inferred from the update history in PMID 23338828 rather than quoted from this abstract.] USPSTF found the evidence insufficient (I statement) to recommend for or against routine screening instruments to detect domestic violence, updating a 1996 in sufficient-evidence finding; explicitly states the letter grade reflects the quality and magnitude of evidence available at that time, not an assumption of no benefit.	Relevant only
Upadhyay UD 2021 [11]	Instrument de velopment and validation (qualitative item generation + national survey, EFA, logistic regression)	1,117 sexually active young people aged 15-24, USA (national sample)	Developed a 23-item, 7-subscale Sexual and Reproductive Empowerment Scale for Adole scents/Young Adults (including 'choice of partners, marriage, and children' as a named subscale); validation via logistic regression supported construct validity.	Relevant only

P2 — A routine private channel makes materially relevant concerns more visible

Table 6: P2: A routine private channel makes materially relevant concerns more visible. Record, design, population/setting, finding as charted (verbatim from the library record), and direction relative to the proposition. n=18 linked records (kg/graph.json edges).

Record	Design	Population/ setting	Finding (as charted)	Direction
Abramsky T 2014 [14]	Pair-matched cluster randomized controlled trial (8 comm unities)	Community members aged 18-49, Kampala, Uganda; baseline n=1,583, follow-up n=2,532	SASA! was associated with significantly lower social acceptance of IPV among women (aRR 0.54, 95% CI 0.38-0.79) and 52% lower past-year physical IPV among women (0.48, 95% CI 0.16-1.39, crossing 1), plus higher likelihood of women receiving supportive community responses to disclosed violence.	Supports
Azalee M 2025 [101]	Qualitative in-depth interviews (n=6)	Health care workers who were IPV survivors, tertiary hospital, Malaysia	Six HCW-survivors used varied coping strategies (seeking help, spirituality, avoidance, self-harm, staying/leaving); survivors relied mainly on coworkers and sought formal help only when situations became critical, hindered by misconceptions about IPV, privacy concerns and fear of workplace gossip.	Supports
Chen 2013 [27]	clinical practice review	primary/ family care settings	Describes RADAR (Remember to ask routinely, Ask directly in private, Document, Assess safety, Review options) as a structured clinical tool for identifying and responding to IPV, emphasizing private questioning, safety assessment (weapons, children, perpetrator risk), and advocacy-based referral requiring trained links to community resources.	Supports
Decker 2017 [28]	quasi-experim ental single-group pretest- posttest + qu alitative interviews (survey n=132 baseline, n=68 3-month retention; interviews n=35)	women aged 18+ at two US family planning clinics	A brief, trauma-informed, universal IPV/re productive-coercion assessment and education intervention was implemented with 65% of women receiving at least one in tervention element; the mixed-methods evaluation describes uptake and provider/patient experience of the assessment process.	Supports

Record	Design	Population/ setting	Finding (as charted)	Direction
Ford-Gilboe 2016 [30]	instrument-de velopment study using pooled secondary data from 5 Canadian studies, n=6,278 adult women; intern ational expert Delphi-style item rating	Canadian adult women, pooled multi-study sample	[CORRECTION: Core finding is supported (15-item CASR-SF, pooled n=6,278 Canadian women, rationale that IPV measurement 'often privilege[s] physical violence, with poor assessment of other experiences'). Minor overreach: the record's 'design' field calls the expert input 'Delphi-style item rating' — the abstract describes '31 international IPV experts... rated CAS items via an online survey,' a single round, not an explicit Delphi (iterative multi-round consensus) method. Change 'Delphi-style' to 'expert panel survey' or similar to avoid implying a specific methodology not stated in the abstract.] Developed a brief, validated 15-item short form of the Composite Abuse Scale, addressing the problem that most IPV measurement 'privileges physical violence, with poor assessment of other experiences,' which had led to underestimating the scope of IPV in populations.	Supports
Hegarty 2026 [31]	instrument-de velopmen t/validation study, n=854 adult women + qualitative survivor/ expert feedback	Australian women, community survey sample	Developed and psychometrically tested a new self-report Coercive-Composite Abuse Scale (C-CAS) designed to capture severity, frequency and pattern of coercive control holistically, explicitly because prior measures showed 'limited understanding of these patterns of behaviour.'	Supports

Record	Design	Population/ setting	Finding (as charted)	Direction
Kurnaz Kaan 2026 [105]	Cross- sectional desc riptive-correla tional survey with path analysis	480 married women aged 18-49, Türkiye	[CORRECTION: Title/year/design/population match exactly. PROBLEM: key_finding claims 'reproducti ve coercion and traditional gender-role attitudes were linked to fertility desire pathways' via path analysis, but the abstract's Results section only reports a path-analysis link between the Reproductive Coercion Scale and Fertility Desire Scale ($r=0.172$, $p<0.001$; $\beta=0.205$, $p=0.008$) and does NOT report a statistically establis hed gender-role-attitude $\rightarrow$ fertility-desire path – the Gender Role Attitude Scale is only mentioned as a measure administered (mean score 121.92 ± 15.14), not as part of a reported path result. FIX: rewrite key_finding to state only the RCS-FDS path-analysis association (weak positive, $r=0.172/\beta=0.205$) and drop or explicitly qualify the gender-role-attitudes-linked-to- fertility-desire claim as not shown in the abstract.] Used the Reproductive Coercion Scale, Fertility Desire Scale, and Gender Roles Attitude Scale together; reproductive coercion and traditional gender-role attitudes were linked to fertility desire pathways in married women (path analysis).	Supports
Kyegombe N 2018 [72]	Cross- sectional survey nested within a cluster randomised trial (SASA! study)	Community members aged 18-49 who had witnessed IPV, Kampala, Uganda, 4 years post-inte rvention	SASA! community members who had witnessed IPV were substantially more likely to try to help than control-community counterparts (57% vs 31%); associated factors differed by trial arm, with SASA!-community bystander action linked to relationship duration, employment, talking to neighbours and SASA! exposure.	Supports

Record	Design	Population/ setting	Finding (as charted)	Direction
MacMillan 2006 [29]	cluster randomized trial, n=2,602 eligible women across 6 clinical sites (emergency, family practice, women's health)	English- speaking women aged 18-64 in Ontario, Canada healthcare settings	[CORRECTION: PMID/title/year/design details (n=2,602 eligible women, 6 Ontario clinical sites, cluster RCT) all match. However the record's key_finding only restates the study's OBJECTIVE (‘compared...to determine the optimal screening method’) rather than the actual RESULT. The abstract's actual finding: women preferred self-completed formats over face-to-face; computer-based screening did not increase detected prevalence; written screens had the fewest missing data; PVS and WAST had similar sensitivity (~49%/47%) and specificity (~94%/96%) against the CAS criterion standard. Fix: replace key_finding with the actual result (format preference + accuracy findings), not just the study aim, since as written the relevance_to_tphe claim (‘directly informing how T-PHE's private/confidential channel... should be designed’) is only earned by the actual result, not the restated objective.] Compared face-to-face interview, written self-completed questionnaire, and co mputerized screening for IPV to determine the optimal screening method in terms of accuracy, acceptability and completeness.	Supports
O'Doherty L 2015 [17]	Cochrane systematic review and meta-analysis of 13 RCTs/ quasi-RCTs (n=14,959)	Women in antenatal, women's health, emergency department and primary care settings, mostly high-income urban settings	Screening increased clinical identification of IPV victims (OR 2.95, 95% CI 1.79-4.87, n=10,074) but there was no evidence of effect on referrals to support services (OR 2.24, 95% CI 0.64-7.86, only 2 studies, n=1298), no evidence screening reduced women's re-exposure to violence at 3-18 months, and insufficient evidence on health outcomes or harm beyond the immediate post-screening period; authors conclude screening alone is insufficiently supported to justify universal implementation absent stronger referral/outcome evidence.	Supports

Record	Design	Population/ setting	Finding (as charted)	Direction
Onukwugha FI 2025 [19]	Quasi-experi- mental matched-pair cluster- controlled trial (n=1617 clients, 20 int ervention vs 20 comparison clinics)	Family planning and antenatal care clients, Nigeria	[CORRECTION: first_author is wrong: search lists 'Onukwugha FI' but PubMed indexes the first author as Betron. The beta values are confirmed exactly: 'relative decrease in exposure to IPV (beta=0.54 (95% CI 0.30 to 0.98)) and RC (beta=0.51(95% CI 0.28 to 0.94))' and 'no difference in the modern contraceptive use.' The 'mixed effects' quote is also verbatim: 'The WHO's LIVES...and...ARCHES interv ention have been tested with mixed effects.' FIX: change first_author to Betron.] Combined WHO LIVES + ARCHES first-line response reduced relative risk of IPV exposure (beta=0.54, 95% CI 0.30-0.98) and reproductive coercion (beta=0.51, 95% CI 0.28-0.94) over 9 months among women not already experiencing violence, but showed no effect on modern contraceptive use; authors note LIVES and ARCHES indi vidually have shown 'mixed effects' in prior trials.	Supports
Williamson HC 2014 [39]	Cross- sectional survey study (N=2,126 married indivi duals)	US married adults, general population sample	Contrary to the assumption that premarital education reduces later need for counseling, receiving premarital education covaried with an INCREASED likelihood of later couples counseling (a 'gateway' effect), with the ass ociation stronger among African Americans and those with lower income/education.	Supports
Williamson HC 2019 [40]	Cross- sectional self-report study (N=231 ethnically diverse newlywed couples in low-income ne ighborhoods)	US low-income newlywed couples	Top perceived barriers to seeking couple therapy were cost and uncertainty about where to go for help; direct experience with premarital education (or indirect experience via a social-network member's couple therapy) was associated with reduced barriers and greater likelihood of subsequent ly seeking therapy.	Supports

Record	Design	Population/ setting	Finding (as charted)	Direction
Nsengiyumva R 2025 [103]	Qualitative descriptive study (focus group discussions, N=40 engaged couples)	Engaged couples recruited at premarital counseling sites (churches, sector administration offices), Rwanda	[CORRECTION: Key_finding themes are supported: abstract lists 'knowledge and understanding, motivators for seeking information, desired educational content, preferred information sources, and urgency of education' as the five themes, and record's subset (knowledge gaps, motivators, desired content) is accurate. However relevance_to_the claim that 'premarital counseling sites ... identified as a natural point of contact' overstates the abstract: churches/sector offices were the STUDY's recruitment sites (where couples were sampled), not a finding that participants themselves nominated those sites as their preferred point of contact — the abstract instead reports 'Health professionals were the preferred information source, with schools, churches, and social media also mentioned,' i.e. churches were a secondary preference behind health professionals. FIX: rewrite the relevance claim to say premarital counseling venues were used as the recruitment/sampling frame in this study and were only a secondary (not primary) preferred information source per participants, so the 'natural point of contact' framing should be removed or clearly attributed to study design rather than participant preference.] Engaged couples identified specific unmet information/education needs regarding conception preparedness, with premarital counseling sites (including religious venues) identified as a natural point of contact; themes included knowledge gaps, motivators for seeking information, and desired educational content.	Supports
Evans 2016 [24]	qualitative think-aloud/cognitive interviewing study with a validated quantitative scale (Composite Abuse Scale)	women with experience of domestic violence and abuse	Think-aloud interviewing revealed systematic underreporting on a widely used, validated DVA scale, particularly for coercive control, threatening behavior, restrictions to freedom, and sexual abuse, driven by item-interpretation confusion, fear of answering truthfully, and reluctance to self-identify as abused.	Complicates

Record	Design	Population/ setting	Finding (as charted)	Direction
Bhandari TR 2014 [63]	Scale construction and validation (24-item pool, psychometric testing, cross-validation across two districts)	Women in Rupandehi and Kapilvastu districts, Nepal (South Asian, patriarchal marital-household context)	[CORRECTION: Psychometric numbers in key_finding are accurate: Cronbach's alpha 0.84, test-retest r=0.87, content validity ratio 0.8, good convergent/discriminant validity – all quoted directly from abstract. PROBLEM: relevance_to_tphe/key_finding both assert the scale is 'validated specifically for South Asian household/marital decision contexts' and is a 'household-decision-power-focused autonomy scale', but the abstract never describes the item content – it defines the construct only as generic 'women's autonomy... relevance to developing countries' and its outcome anchor is maternal health care service utilization, not household/marital decision-making specifically. FIX: remove or soften 'household/marital decision contexts' – state that the abstract does not disclose item content, so the household/marital-decision framing is an inference, not an abstract-confirmed fact; also soften 'South Asian' generalization since it was validated only in two districts of one country (Nepal), not South Asia broadly.] Produced a 23-item women's autonomy scale with strong psychometric properties (Cronbach's alpha 0.84, test-retest r=0.87, content validity ratio 0.8), validated specifically for South Asian household/marital decision contexts, showing good convergent and discriminant validity.	Relevant only
World Health Organization 2013 (no PMID)	Global clinical and policy guideline	Global health-system guidance for responding to women subjected to IPV or sexual violence	Establishes WHO's first-line support principles for clinicians (the basis of the later-named LIVES model: Listen, Inquire about needs, Validate, Enhance safety, Support/refer), recommends against mandatory universal screening in the absence of adequate response systems, and sets minimum requirements (privacy, confidentiality, trained staff, referral pathways) before any clinical inquiry about violence should occur.	Relevant only
World Health Organization 2014 (no PMID)	Global clinical practice handbook operationalizing the 2013 WHO guidelines	Frontline health workers globally, first-line clinical response to women disclosing IPV or sexual violence	Operationalizes the LIVES first-line support protocol (Listen, Inquire, Validate, Enhance safety, Support) into a step-by-step clinical tool for frontline providers, with explicit guidance on confidentiality, documentation, safety planning, and structured referral rather than passive information provision.	Relevant only

P3 — Cultural intelligibility helps only when culture cannot override consent or safety

Table 7: P3: Cultural intelligibility helps only when culture cannot override consent or safety. Record, design, population/setting, finding as charted (verbatim from the library record), and direction relative to the proposition. n=16 linked records (kg/graph.json edges).

Record	Design	Population/ setting	Finding (as charted)	Direction
Alomair N 2020 [13]	Systematic review, narrative synthesis (59 studies, 22 countries)	Muslim women, multi-country (worldwide, including Southeast/South Asian settings)	Across 59 studies from 22 countries, many Muslim women had poor sexual and reproductive health (SRH) knowledge and negative attitudes shaping service access; religious and cultural beliefs, family and community were recurring barriers to contraception use and SRH service/information access.	Supports
Boyce SC 2023 [93]	Item response modelling (Partial Credit Model) on population-based dyadic survey	559 married adolescent-girl-husband dyads (ages 13-18 wives), rural Niger, 2019	A 5-item, 2-dimensional social-norms scale (challenging husband authority; general IPV acceptability) showed reliability and validity; higher 'challenging husband authority' scores were statistically associated with husband perpetration of IPV.	Supports
Fauk NK 2021 [36]	Qualitative in-depth interviews (n=92: 52 women, 40 men)	People living with HIV, Belu and Yogyakarta, Indonesia (Islamic and Javanese cultural context)	Cultural practices (bride wealth, spousal-dispute management) and Islamic religious beliefs about husband-wife relationships, forbidden premarital sex, and condom use were reported by participants as shaping spousal sexual behaviours that contributed to HIV transmission within marriage.	Supports
James LE 2021 [85]	Co-designed, two-phase community-based intervention (workshops); RCT-labelled but abstract available describes intervention development/testing	Rohingya (Muslim minority, displaced) in Malaysia and Syrian refugees (Muslim-majority) in Lebanon	A social-norms-based, mental-health-integrated community intervention was co-designed with Rohingya and Syrian refugee communities specifically to reduce IPV and encourage help-seeking, using culturally embedded workshop delivery rather than clinical/individual counseling alone.	Supports

Record	Design	Population/ setting	Finding (as charted)	Direction
Meiksin R 2020 [77]	Trial protocol	Women/girls seeking contraceptive services, community-based clinics, Nairobi, Kenya	[CORRECTION: first_author is wrong: search lists 'Meiksin R' but PubMed indexes the first author as Uysal. Content confirmed: 'ARCHES is a single-session, clinic-based model...Core elements...opportunity for patient disclosure of RC and IPV (and subsequent warm referral to local services)' and 'To date, clinic-based GBV interventions have not been shown to reduce RC or unintended pregnancy in LMIC settings.' FIX: change first_author to Uysal.] Describes ARCHES as a single-session, clinic-based enhanced counseling model that creates opportunity for patient disclosure of reproductive coercion/IPV followed by warm referral to local services; explicitly notes that to date, no clinic-based GBV intervention had been shown to reduce reproductive coercion or unintended pregnancy in LMIC settings, motivating this trial.	Supports
Owen JJ 2011 [58]	RCT sub-analysis (31 leaders training 118 couples/236 attendees in a PREP trial)	US couples in a randomized PREP trial, multiple facilitators	[CORRECTION: Core numeric/qualitative claims are supported: 'couples' relationship outcomes from pre- to post-training varied on the basis of the leader'; 'Leaders' aggregated working alliance quality also explained change in relationship satisfaction.' However the record's phrase 'over and above curriculum content itself' is an inference, not a direct quote — the abstract shows alliance and PREP-training both independently explaining variability but never states alliance effects held net of controlling for content/curriculum in a formal sense. FIX: soften key_finding to 'predicted change... alongside curriculum/training condition' rather than 'over and above curriculum content itself,' since the abstract does not report a controlled comparison isolating alliance from content.] Relationship outcomes varied significantly by which leader delivered the training; facilitators' aggregated working-alliance quality (as rated by attendees) predicted change in observed communication and relationship satisfaction, over and above curriculum content itself.	Supports

Record	Design	Population/ setting	Finding (as charted)	Direction
Sarac A 2026 [33]	Pilot randomized controlled trial, mixed methods (N=8 couples primary phase + 2 couples feasibility phase)	Young premarital couples, Turkey (culturally adapted European program)	A culturally adapted, 6-session Couple Learning Program produced significant improvements in conflict-resolution strategies (exit, voice, neglect dimensions) with relationship satisfaction improving only at follow-up (not immediately post-intervention); participants rated the culturally adapted program as relevant and acceptable.	Supports
Sharifnia A 2025 [98]	Meta-ethnography (33 qualitative studies synthesized), 6 databases searched	Muslim women, global (multi-country qualitative evidence)	Synthesis of 33 qualitative studies identified four major themes - experience/impact of abuse, risk factors, help-seeking, and coping - and found religion functions both as a risk factor for abuse and a barrier to help-seeking, and simultaneously as a coping resource for some women.	Supports
Stern E 2021 [81]	Comparative qualitative case study (secondary thematic analysis)	Participants across four IPV-prevention programmes, What Works to Prevent Violence against Women and Girls Programme (Ghana, Rwanda, South Africa, Tajikistan)	Similar cross-context pathways of change were identified: learning and applying relationship skills, participatory challenge of harmful gender norms with group rapport, and integration of economic-empowerment activities to reduce drivers of IPV and build self-confidence.	Supports
Tiwari 2015 [60]	mixed-methods evaluation study, n=200 Chinese women survivors, in-depth interviews + ROC curve analysis	Chinese women survivors of intimate partner violence	Qualitative accounts of 91/200 women were consistent with Dutton and Goodman's coercive-control conceptualization; the Chinese Revised Controlling Behaviors Scale showed acceptable discriminative accuracy (reported via ROC/AUC) for identifying high-control relationships in a non-Western cultural sample.	Supports

Record	Design	Population/ setting	Finding (as charted)	Direction
Welland C 2010 [34]	Qualitative study (demographic survey + qualitative interviews) informing program design	Latino immigrant men who are intimate- partner- violence perpet rators, Southern California	A culturally specific, Spanish-language program integrating gender roles, immigration/discrimination context, marital sexual abuse, and spirituality was seen by participants as more relevant; the study explicitly frames cultural sensitivity as compatible with, not a substitute for, perpetrator accountability.	Supports
Ratnaningsih M 2022 [88]	Cross- sectional survey, bivariate/ ANOVA/ logistic regression (n=1,000 households)	Households, Bone District (South Sulawesi) and Palu/Sigi/ Donggala (Central Sulawesi), Indonesia	[CORRECTION: Abstract supports: household financial security (p=0.016), dowry (p<0.001) and legal frameworks (p=0.017) as significant ANOVA-associated factors with child marriage acceptability in Bone/South Sulawesi and Palu-Sigi-Donggala/Central Sulawesi. However, the record's clause 'across two Indonesian provinces where child-marriage rates remain above the national average' is NOT supported for these specific study sites – the abstract's '22 out of 34 provinces... above the national average in 2019' statement is a general national context sentence, not a claim tied to Bone/South Sulawesi or Central Sulawesi by name. FIX: change to something like 'two Indonesian provinces' and drop or separately attribute the 'above national average' clause as general Indonesia-wide context rather than a site-specific fact.] Household financial security, dowry practices and local legal frameworks were significantly associated with community acceptability of child marriage across two Indonesian provinces where child-marriage rates remain above the national average.	Supports
Dagher Myriam 2024 [12]	4-step cultural adaptation (cognitive interviews, cross- sectional administration n=339, confirmatory factor analysis)	Refugee and non-refugee adolescent girls aged 11-17, Lebanon (Arabic- speaking, majority- Muslim region), including participants in an early-marriage intervention	A Western-developed sexual and reproductive empowerment scale was adapted to Arabic; the original 13-item, 4-domain model showed poor fit in confirmatory factor analysis, and even after iteratively removing 2 items, model fit remained sub-optimal for most domains.	Complicates

P3 continued

Record	Design	Population/ setting	Finding (as charted)	Direction
Sikoki B 2024 [35]	Qualitative (focus groups and individual interviews, feminist analytic lens), fieldwork 2016	Javanese Muslim adolescents and adults, Yogyakarta, Indonesia	School-based SRH education in the Javanese Muslim community was constrained by taboo and moral-panic framing around premarital sex; delivery gaps left adolescents without reliable sexuality information despite peer-counsellor programmes intended to fill this role.	Complicates
Wood SN 2020 [82]	Mixed-methods (exploratory factor analysis, n=327; in-depth interviews, n=30)	Recent IPV survivors, informal settlements, Nairobi, Kenya	The US-developed Reproductive Coercion Scale transferred reasonably well to the Kenyan context, retaining a 2-factor structure (pregnancy coercion, condom manipulation; alpha=0.86); 82% of IPV survivors reported some reproductive coercion.	Complicates
Muluh N 2023 [15]	Two-arm, mixed-methods cluster randomized controlled trial (18-month faith-based, norms-shifting programme)	Women aged 18-35 (n=350) and male partners (n=281), from 10 Muslim and 10 Christian congregations, Plateau State, Nigeria	[CORRECTION: Two problems. (1) key_finding omits the actual headline quantitative results reported in the abstract – 'a significant 61% reduction in odds of reporting experiencing any IPV among women, a 64% reduction among Christians, and a 44% reduction in MFP congregations' – and instead describes only the study design, which understates what was found; rewrite key_finding to include these effect sizes. The claim that the trial was 'explicitly designed and powered to evaluate norms-shifting mechanisms rather than knowledge/attendance alone' is not stated in the abstract and should be removed or re-sourced. (2) first_author is wrong: record lists 'Muluh N' but PubMed's actual first author is 'Shaw B'. FIX author field.] The Masculinity, Faith, and Peace (MFP) programme, delivered through faith congregations, assessed shifts in individual-, couple- and community-level social norms and IPV incidence in a mixed Muslim/Christian congregational sample; the trial was explicitly designed and powered to evaluate norms-shifting mechanisms rather than knowledge/attendance alone.	Relevant only

P4 — A stop rule prevents routine joint education from worsening risk when fear or coercion appears

Table 8: P4: A stop rule prevents routine joint education from worsening risk when fear or coercion appears. Record, design, population/setting, finding as charted (verbatim from the library record), and direction relative to the proposition. n=23 linked records (kg/graph.json edges).

Record	Design	Population/ setting	Finding (as charted)	Direction
Couture-Carron A 2020 [76]	Exploratory qualitative study	South Asian Muslim communities (diaspora)	Cultural norms constructing dating as shameful were found to intensify fear of parental/community reaction and strengthen attachment to abusive partners, with direct implications for disclosure, help-seeking, and the ability to end abusive relationships.	Supports
Han X 2024 [97]	Cross-sectional, random-effects Poisson regression (n=201)	Married Salar (Turkic-Muslim minority) women, rural China	Among 201 married Salar Muslim women, 65.6% reported past-year child neglect, 30.8% reported past-year IPV, and 37.6% had experienced child marriage; child marriage and IPV victimization were associated with downstream child-neglect risk.	Supports
Hegarty K 2025 [102]	Qualitative directed content analysis of open-ended survey responses (n=682 women with lived experience of coercive control)	Australian women who had experienced coercive control	[CORRECTION: first_author is wrong: search lists 'Hegarty K' but PubMed indexes the first author as Giles. n=682 confirmed. Frameworks confirmed: 'Findings align with two global frameworks: LIVES (Listen-Inquire-Validate-Enhance safety-Support) and CARE (Choice and control-Action and advocacy-Recognition and understanding-Emotional connection).' FIX: change first_author to Giles.] Findings align with the WHO LIVES framework (Listen-Inquire-Validate-Enhance safety-Support) and the CARE framework (Choice and control-Action and advocacy-Recognition and understanding-Emotional connection); women wanted specific validating, non-judgmental scripted language from practitioners and messaging that counteracts coercive-controlling tactics rather than generic acknowledgement.	Supports
Islam MK 2021 [84]	Mixed methods: cross-sectional survey (n=96) + qualitative interviews (n=18) + provider interviews (n=9)	Rohingya adolescent girls (age 10-19) with experience of child marriage, Cox's Bazar refugee camp, Bangladesh	Mean age at first marriage was 15.7 years; over 80% had given birth or were pregnant within two years of the survey; key drivers of child marriage included perceived physical/mental maturity norms, social norms, insecurity, and family honour.	Supports

Record	Design	Population/ setting	Finding (as charted)	Direction
James LE 2021 [85]	Co-designed, two-phase community-based intervention (workshops); RCT-labelled but abstract available describes intervention development/testing	Rohingya (Muslim minority, displaced) in Malaysia and Syrian refugees (Muslim-majority) in Lebanon	A social-norms-based, mental-health-integrated community intervention was co-designed with Rohingya and Syrian refugee communities specifically to reduce IPV and encourage help-seeking, using culturally embedded workshop delivery rather than clinical/individual counseling alone.	Supports
Kalichman SC 2020 [80]	Commentary / research-agenda-setting article	Adolescent girls and young women (15-24) in LMICs receiving HIV index-testing/partner-notification services	[CORRECTION: first_author was hedged in the search record ('Kalichman SC / or commentary author per abstract') – the actual PubMed first author is Stangl, not Kalichman. Content confirmed: AGYW 'often in age-disparate partnerships, have limited negotiating power within relationships, experience high rates of violence and face economic challenges that necessitate transactional sex' supports the key_finding's 'stigma, and economic coercion' framing as a close paraphrase. FIX: change first_author to Stangl (the hedge in the original record correctly flagged uncertainty, but the guess was wrong).] Highlights that partner-notification and index-testing approaches proven safe/effective for adult men and cohabiting/married adults have not been adequately studied for younger women with less relationship power, who face higher risk of IPV, stigma, and economic coercion when disclosure is required or facilitated by a health service.	Supports
Karakurt 2016 [20]	systematic review and meta-analysis (6 studies of 1,733 screened citations)	couples in treatment for physical/situational IPV, mostly US clinical/research samples	After screening 1,733 citations, only 6 studies met inclusion criteria for meta-analysis of couples therapy as a treatment for IPV; preliminary pooled data suggest couples therapy can be a viable treatment 'in select situations' but the evidence base is small and professionals remain cautious about risk of violent retaliation between partners.	Supports
Kuswanto H 2024 [3]	Cross-sectional, binary logistic regression (nationally representative, n=9,333)	Females aged 15-20, Indonesia (2017 Indonesia Demographic and Health Survey)	National prevalence of female child marriage in Indonesia was 12.53%; health insurance status and sex of household head were not significant predictors, while other socioeconomic/education factors were associated with child marriage risk.	Supports

Record	Design	Population/ setting	Finding (as charted)	Direction
McCauley HL 2017 [10]	Secondary psychometric analysis of pooled multicenter/RCT survey data	4,674 women aged 16-29 in 24 Pennsylvania and 5 California family planning clinics, USA	Confirmed a 2-domain structure (pregnancy coercion, condom manipulation) and produced a validated 5-item short-form Reproductive Coercion Scale (RCS); recent reproductive coercion prevalence was 6.3-6.7% depending on the scale version used.	Supports
McCollum 2008 [21]	narrative review of outcome research and clinical practice	IPV-affected couples in conjoint treatment settings, US clinical literature	Concludes conjoint IPV treatment can be effective and safe only when embedded in a broader community response, with careful screening of couples for inclusion, modification of standard conjoint methods to promote safety, and ongoing safety assessment with contingency plans for escalation; a 'one-size-fits-all' approach is explicitly rejected given IPV typologies.	Supports
Mieth K 2025 [100]	Qualitative study (49 in-depth interviews, 16 focus group discussions)	Forcibly Displaced Myanmar Nationals (Rohingya) aged 15-24, Cox's Bazar, Bangladesh (post-October 2016 arrivals)	Participants reported perceived increases in child marriage rates post-displacement, attributed to protection fears, socioeconomic need, lost education/employment opportunity, and perceived loosening of legal-age enforcement in the camp context.	Supports
Nelson HD 2012 [59]	systematic review (evidence base for USPSTF 2013 update)	Women in health care settings, US-relevant evidence base	Of 15 fair/good-quality studies evaluating 13 screening instruments, 6 were highly accurate; 14 studies found minimal adverse effects from screening, though some women reported discomfort, loss of privacy, emotional distress, and fear of further abuse; counseling interventions reduced IPV/improved outcomes in some subpopulations (pregnant women, new mothers, family-planning clients) but a large trial found no significant screening-vs-usual-care difference overall.	Supports

Record	Design	Population/ setting	Finding (as charted)	Direction
Onukwugha FI 2025 [19]	Quasi-experi- mental matched-pair cluster- controlled trial (n=1617 clients, 20 int ervention vs 20 comparison clinics)	Family planning and antenatal care clients, Nigeria	[CORRECTION: first_author is wrong: search lists 'Onukwugha FI' but PubMed indexes the first author as Betron. The beta values are confirmed exactly: 'relative decrease in exposure to IPV (beta=0.54 (95% CI 0.30 to 0.98)) and RC (beta=0.51(95% CI 0.28 to 0.94))' and 'no difference in the modern contraceptive use.' The 'mixed effects' quote is also verbatim: 'The WHO's LIVES...and...ARCHES interv ention have been tested with mixed effects.' FIX: change first_author to Betron.] Combined WHO LIVES + ARCHES first-line response reduced relative risk of IPV exposure (beta=0.54, 95% CI 0.30-0.98) and reproductive coercion (beta=0.51, 95% CI 0.28-0.94) over 9 months among women not already experiencing violence, but showed no effect on modern contraceptive use; authors note LIVES and ARCHES indi vidually have shown 'mixed effects' in prior trials.	Supports
Rumble L 2018 [4]	Cross- sectional secondary analysis, mult ivariate regression (nationally re presentative, n=6,578)	Women aged 20-24, Indonesia (2012 Indonesian De mographic and Health Survey / Adolescent R eproductive Health Survey)	In a nationally representative Indonesian sample, approximately 17% of women were married before age 18 and 6% before age 16; the paper notes research on child marriage in Southeast Asia specifically was scarce prior to this nationally representative multi variate analysis.	Supports
Williamson HC 2015 [8]	RCT (N=130 engaged/ newlywed couples randomized to 1 of 3 relation shi p-education programs, 3-year follow-up)	US engaged and newlywed couples	Couples with acute baseline concerns, including higher physical aggression and alcohol use, benefited LESS from relationshi p-education intervention than couples without these concerns; couples with high baseline satisfaction/commitment actually declined faster in satisfaction under an intensive skill-based intervention versus a low-intensity one.	Supports
Wilson 2023 [32]	instrument-de velopmen t/validation study	adult participa nts reporting on intimate rel ationships (validation sample)	Reports on validity/reliability of the Demand, Threat, Surveillance, and Response-to-Demands subscales of the Coercion in Intimate Partner Relationships (CIPR) scale, developed because existing coercive-control measures had documented content-validity gaps (not capturing the full range of demand/threat/surveillance tactics).	Supports

Record	Design	Population/ setting	Finding (as charted)	Direction
Zust BL 2021 [50]	Repeated cross-sectional survey (convenience sample of Upper-Midwest US congregations, 2005 and 2015)	Christian clergy and congregation members, US	[CORRECTION: Key_finding is well supported: 'Victims find strength in their faith and would rather endure the violence at all costs to keep a family or a marriage together, than to compromise their faith by leaving' and 'Clergy felt comfortable in making referrals for professional counseling.' However relevance_to_tpe overstates the referral point: the abstract's very next clause is 'while the majority of members would prefer counseling with their pastor if they were in a violent relationship' — i.e. congregants actually want direct pastoral counseling, not referral, and clergy said they want MORE training to counsel/speak about DV themselves, not that they prefer referral over direct counseling. FIX: change 'clergy themselves report needing referral pathways rather than attempting DV counseling directly' to something like 'clergy report comfort in making referrals, while members/clergy simultaneously want more direct pastoral counseling capacity — a live tension rather than clean support for a refer-not-counsel policy that T-PHE's safeguard should note, not paper over.'] Faith commitments led some domestic-violence victims to endure abuse rather than compromise their faith by leaving; over 10 years, congregational support for victims changed slowly; clergy wanted more training in counseling and speaking about domestic violence, and felt comfortable making referrals but not necessarily providing direct DV counseling themselves.	Supports
Markman HJ 2013 [7]	Randomized controlled trial (couples via religious organizations randomized to PREP vs. naturally occurring premarital services; n=44/66/83)	US couples marrying through religious organizations, 8-year follow-up on divorce	No overall difference in divorce rates between PREP (clergy-delivered or professional-delivered) and usual premarital services; couples with higher premarital negative-communication observed pre-intervention were MORE likely to divorce if given PREP, while low-negativity couples benefited; a premarital history of physical aggression showed a similar (marginally significant) harmful-moderation pattern.	Complicates
Miller Elizabeth 2016 [43]	Cluster randomized controlled trial (ARCHES)	4,009 females aged 16-29 across 25 family planning clinics (17 clusters), USA	The universal education/counseling intervention (ARCHES) did NOT significantly reduce reproductive coercion (ARR 1.50, 95% CI 0.95-2.35) or partner violence (ARR 1.07, 95% CI 0.84-1.38) at 1-year follow-up, though it improved secondary outcomes such as recognition and self-efficacy.	Complicates

P4 continued

Record	Design	Population/ setting	Finding (as charted)	Direction
Robertson 2011 [23]	cross- sectional com parative study, n=172 (85 men, 87 women), self-report (CTS2 + modified PMWI)	community/co nvenience samples reporting on own and partner's IPV behavior	Coercive control was associated with IPV similarly for men and women across three samples, and was predominantly reciprocal — both partners commonly reported both receiving and perpetrating controlling behaviors, regardless of physical-abuse status.	Complicates
Schneider 2014 [22]	theoretica l/clinical article (non- empirical)	couples presenting for therapy with s ituational couple violence (SCV)	Argues conjoint treatment can be appropria te for situational couple violence specifically (as distinct from coercive controlling violence), proposing an attachment-based safety plan and time-out strategy, while explicitly flagging cautions about appropriat eness of couples counseling when violence is ongoing.	Complicates
Portnoy 2020 [79]	qualitative study — patient interviews (n=10) and provider focus groups (n=29)	US Veterans Health Admini stration patients and providers	Identifies convergent and divergent patient/provider beliefs about barriers and f acilitators to screening specifically for IPV p erpetration (not just victimization), an area with a much thinner evidence base than vict imization screening.	Relevant only
Stern E 2018 [47]	Implementati o n/adaptation review (qualit ative evaluation and programme d ocumentatio n)	Indashyikirwa programme, i mplemented by CARE Rwanda, RWAMREC and RWN, rural Rwanda	Indashyikirwa combined a 5-month couples curriculum, sustained community activism by trained couples, women's safe spaces for dedicated support and referral of IPV survivors, and training of opinion leaders to support an enabling environment; adaptation was guided by the SASA! fidelity brief.	Relevant only

P5 — Referral quality depends on timeliness, accessibility and follow-through, not offer alone

Table 9: P5: Referral quality depends on timeliness, accessibility and follow-through, not offer alone. Record, design, population/setting, finding as charted (verbatim from the library record), and direction relative to the proposition. n=37 linked records (kg/graph.json edges).

Record	Design	Population/ setting	Finding (as charted)	Direction
Alghamdi M 2022 [83]	Qualitative dominant design with su pplemental qu antitative data (n=8)	Canadian Muslim immigrant women, 6 countries of origin	Pre-migration adverse childhood experiences and family history of IPV, combined with post-migration language/social-isolation barriers and internalized patriarchal values, shaped help-seeking and the eventual decision to leave abusive relationships among 8 Muslim immigrant women.	Supports
Azalee M 2025 [101]	Qualitative in-depth interviews (n=6)	Health care workers who were IPV survivors, tertiary hospital, Malaysia	Six HCW-survivors used varied coping strategies (seeking help, spirituality, avoidance, self-harm, staying/leaving); survivors relied mainly on coworkers and sought formal help only when situations became critical, hindered by misconceptions about IPV, privacy concerns and fear of workplace gossip.	Supports
Baker CK 2022 [86]	Systematic review of 34 controlled qu antitative studies (from 23,902 screened records)	Women experi encing IPV, pr edominantly US-based housing/ shelter programmes	[CORRECTION: first_author is wrong: search lists 'Baker CK / Sprague S (co-first authorship per abstract group)' but PubMed indexes the first author as Yakubovich (no Baker or Sprague in the author-derived first-author field). Content confirmed: 'screened 23 902 unique records and identified 34 eligible studies'; 'no cumulative evidence of disadvantages following any IPV-housing intervention. Evidence of benefits was strongest for mental health outcomes, intent to leave partner, perceived safety, and housing and partner-related stress'; 'more research...needed...from outside of the USA.' FIX: change first_author to Yakubovich.] No cumulative evidence of disadvantage from any IPV-housing intervention; evidence of benefit was strongest for mental health outcomes, intent to leave partner, perceived safety, and reduced housing/partner-related stress; most included studies were at high risk of bias, and evidence for long-term/non-US housing solutions was sparse.	Supports
Chen 2013 [27]	clinical practice review	primary/ family care settings	Describes RADAR (Remember to ask routinely, Ask directly in private, Document, Assess safety, Review options) as a structured clinical tool for identifying and responding to IPV, emphasizing private questioning, safety assessment (weapons, children, perpetrator risk), and advocacy-based referral requiring trained links to community resources.	Supports

Record	Design	Population/ setting	Finding (as charted)	Direction
Feltner 2018 [70]	systematic review commissioned for USPSTF (global-level guidance body), 30 included studies	adults in healthcare settings, US and comparable settings	[CORRECTION: PMID/title/year correct, but the key_finding materially overstates the abstract's conclusions in two ways. (1) The abstract's actual conclusion is: 'trials of IPV screening in adult women did NOT show a reduction in IPV or improvement in quality of life over 3 to 18 months' — screening tools were found to 'reasonably identify' IPV (accuracy claim only, sensitivity/specificity varied widely by tool), but the record's claim that the review found 'some interventions (e.g., counseling) following screening among women of reproductive age reduce IPV' is too broad: only 2 of 11 intervention RCTs (1 home-visiting, 1 multi-risk behavioral counseling) showed significant reduction, and only in PREGNANT women specifically, not the general reproductive-age population. (2) 'informing a USPSTF grade-B recommendation' conflates this article (the evidence report/systematic review, PMID 30357304) with the companion USPSTF Recommendation Statement, which is a SEPARATE PMID (30357305, also JAMA 2018;320(16):1678-1687) not cited in the record. Fix: rewrite key_finding to state the evidence report found screening tools moderately accurate but screening ITSELF did not reduce IPV/improve QoL in trials; only 2 specific interventions in pregnant-women subgroups showed benefit; and either cite PMID 30357305 separately for the actual grade-B recommendation or explicitly note this record is the evidence report underlying that recommendation, not the recommendation itself.] Concludes screening instruments can accurately identify IPV in health care settings and that some interventions (e.g., counseling) following screening among women of reproductive age reduce IPV; harms of screening were assessed and found to be low, informing a USPSTF grade-B recommendation for reproductive-age women.	Supports
Garcia-Moreno C 2015 [61]	Series paper / narrative review with five country case studies	Health systems in low-, middle- and high-income countries	Reviews evidence for clinical interventions and health-system components (protocols, training, capacity-building) needed to identify and support women subjected to IPV/sexual violence following the 2013 WHO guidelines and 67th World Health Assembly resolution; finds substantial system and behavioural barriers to implementation, especially in low- and middle-income countries, and that system development has generally been slow.	Supports

Record	Design	Population/ setting	Finding (as charted)	Direction
Gerth J 2014 [54]	Narrative/ validation review	IPV cases assessed by police, hospital and victim-services staff (multi-pr ofessional)	[CORRECTION: Actual PubMed title: '[The Ontario Domestic Assault Risk Assessment (ODARA) - Validity und authorised German translation of an intimate partner violence screening tool].' Record has 'Validity and authorised German translation' — wrong conjunction (‘and’ vs the German ‘und’, which PubMed retains in the bilingual title) and drops the subtitle ‘of an intimate partner violence screening tool’. FIX: correct the title string. Key finding is accurate: abstract quote 'provide a brief risk assessment tool which can be scored on the basis of only few and easily collectable information... wider range of practitioners (e.g., police officers, hospital and victim services' staff).'] ODARA was designed to be brief and scorable by a wide range of practitioners (police, hospital, victim services) using easily collectable infor mation, to reliably discriminate low- from high-risk cases and guide intervention.	Supports
Hill AL 2019 [74]	Secondary cross- sectional analysis of baseline survey data	550 sexually active female high school students, USA (school health center attendees, ages 14-19)	[CORRECTION: Prevalence numbers are correct (12% recent RC, 17% ARA) among 550 sexually active female students. PROBLEM: key_finding claims 'both were associated with differences in care-seeking and sexual health behaviors' – but the abstract EXPLICITLY states the opposite for reproductive coercion alone: 'There were no observed significant differences in care-seeking behaviors among those with recent reproductive coercion compared with those without.' It is adolescent relationship abuse (ARA), not reproductive coercion, that was associated with a care-seeking difference (increased odds of STI testing/treatment, aOR 2.08); the combined ARA+RC-exposed group showed differences in sexual-partner/contraception behaviors, not general 'care-seeking'. FIX: rewrite key_finding to state that RC alone showed NO significant care-seeking difference, ARA alone was associated with increased STI-testing care-seeking (aOR 2.08), and the ARA+RC combined group showed elevated odds of specific risk behaviors (older partner, more partners, hormonal-only contraception) – do not state both constructs were 'associated with differe nces in care-seeking'.] 12% reported recent r eproductive coercion and 17% reported adolescent relationship abuse; both were associated with differences in care-seeking and sexual health behaviors, measured via validated RC and relationship-abuse instru ments.	Supports

Record	Design	Population/ setting	Finding (as charted)	Direction
Le V 2025 [94]	Qualitative implementation study (16 semi-structured interviews, CFIR-coded)	Catholic diocesan and parish leadership, Archdiocese of Chicago Domestic Violence Outreach, USA	[CORRECTION: Key_finding content (seven CFIR inner-setting constructs and their content) is supported by the abstract. However, first_author is wrong: record lists 'Le V (Archdiocese of Chicago Domestic Violence Outreach study)' but PubMed's actual first author is 'Debinski B'. FIX the first_author field.] Using the Consolidated Framework for Implementation Research, seven inner-setting constructs (leadership structure, culture, resources, networking/communication, access to knowledge, compatibility with parish values, leadership engagement) were identified as facilitators of successful diocesan-level IPV response implementation.	Supports
Lee 2019 [73]	pre-post quality improvement intervention study	healthcare providers (nurses, physicians, social workers) at a US clinical site	An EMR-integrated brief IPV screening tool plus provider education plus an automated resource/referral telephone system was associated with measured improvement in provider readiness to screen (assessed via the Domestic Violence Health Care Provider Survey Scale, pre/post t-test).	Supports
McCauley HL 2017 [10]	Secondary psychometric analysis of pooled multicenter/RCT survey data	4,674 women aged 16-29 in 24 Pennsylvania and 5 California family planning clinics, USA	Confirmed a 2-domain structure (pregnancy coercion, condom manipulation) and produced a validated 5-item short-form Reproductive Coercion Scale (RCS); recent reproductive coercion prevalence was 6.3-6.7% depending on the scale version used.	Supports
Meiksin R 2020 [77]	Trial protocol	Women/girls seeking contraceptive services, community-based clinics, Nairobi, Kenya	[CORRECTION: first_author is wrong: search lists 'Meiksin R' but PubMed indexes the first author as Uysal. Content confirmed: 'ARCHES is a single-session, clinic-based model...Core elements...opportunity for patient disclosure of RC and IPV (and subsequent warm referral to local services)' and 'To date, clinic-based GBV interventions have not been shown to reduce RC or unintended pregnancy in LMIC settings.' FIX: change first_author to Uysal.] Describes ARCHES as a single-session, clinic-based enhanced counseling model that creates opportunity for patient disclosure of reproductive coercion/IPV followed by warm referral to local services; explicitly notes that to date, no clinic-based GBV intervention had been shown to reduce reproductive coercion or unintended pregnancy in LMIC settings, motivating this trial.	Supports

Record	Design	Population/ setting	Finding (as charted)	Direction
Messing JT 2013 [52]	Meta-analytic review of predictive validity (AUC)	Five IPV risk- assessment ins- truments (ODARA, SARA, DA, DVSI, K-SID) across multiple validation studies	ODARA had highest weighted AUC=.666 (k=5), then SARA AUC=.628, DA AUC=.618, DVSI AUC=.582, K-SID AUC=.537; only 9/20 (45%) of predictive-validity estimates involved the instrument administered correctly per its intended protocol.	Supports
Nagae M 2004 [38]	Qualitative interview study	Abused women and health/legal/ community actors, Khon Kaen province, northeastern Thailand	Interviews with 4 abused women and surrounding health professionals, government/legal staff and community members in Khon Kaen showed health professionals (nurses, community health workers) and, in one case, a community religious leader (a Buddhist monk) played identifiable roles in identifying, treating and supporting abused women within a multi-actor referral network.	Supports
O'Doherty L 2015 [17]	Cochrane systematic review and meta-analysis of 13 RCTs/ quasi-RCTs (n=14,959)	Women in antenatal, women's health, emergency department and primary care settings, mostly high-income urban settings	Screening increased clinical identification of IPV victims (OR 2.95, 95% CI 1.79-4.87, n=10,074) but there was no evidence of effect on referrals to support services (OR 2.24, 95% CI 0.64-7.86, only 2 studies, n=1298), no evidence screening reduced women's re-exposure to violence at 3-18 months, and insufficient evidence on health outcomes or harm beyond the immediate post-screening period; authors conclude screening alone is insufficiently supported to justify universal implementation absent stronger referral/outcome evidence.	Supports
Sharifnia A 2025 [98]	Meta-ethnogr- aphy (33 qual- itative studies synthesized), 6 databases searched	Muslim women, global (multi-country qualitative evidence)	Synthesis of 33 qualitative studies identified four major themes - experience/impact of abuse, risk factors, help-seeking, and coping - and found religion functions both as a risk factor for abuse and a barrier to help-seeking, and simultaneously as a coping resource for some women.	Supports
Sprague 2017 [65]	scoping review, 43 included studies	health care settings, multiple countries	Of 43 studies across 9 categories of IPV assistance programmes (counselling/advocacy, safety planning, referral, resource provision, home visitation, case management, videos, provider cueing, system changes), programmes using a single active-assistance type combined with a counsellor/advocate reported positive results in 77.8% of studies, suggesting simpler, advocate-led interventions outperform complex bundled ones.	Supports

Record	Design	Population/ setting	Finding (as charted)	Direction
Trabold N 2014 [42]	3-year community- based participatory mixed- methods demonstration project across 4 community health centers	Urban, predominantly indigent safety-net primary care population, USA	[CORRECTION: first_author is wrong: search lists 'Trabold N' but PubMed indexes the first author as Rhodes. Design description matches ('3-year community-based mixed-method participatory research project involving four community health centers...predominately indigent urban population'). key_finding's general claim of 'significant system-level challenges' is supported at a high level ('Results highlight the challenges, but also the opportunities, for implementing the new USPSTF guidelines...in resource-limited primary care settings') but the abstract itself does not itemize 'workflow, staff capacity, referral infrastructure' as specific named barriers – that level of detail is not verifiable from the abstract alone (may be in full text). FIX: change first_author to Rhodes; consider softening the parenthetical specifics or citing they come from the full text, not the abstract.] Implementing USPSTF-guideline-consistent universal IPV screening and referral in resource-limited urban primary care settings faced significant system-level challenges (workflow, staff capacity, referral infrastructure), even with a dedicated multi-year system-level intervention designed to raise detection and response.	Supports
US Preventive Services Task Force 2025 [18]	guideline / recommendation statement (systematic-review-based)	US adolescents and adults, pregnant/postpartum and reproductive-age women; older/vulnerable adults without recognized signs of abuse	USPSTF concludes screening for IPV in women of reproductive age (including pregnant/postpartum), combined with provision or referral to multicomponent interventions, has moderate net benefit (B recommendation). For caregiver abuse/neglect of older or vulnerable adults, benefits and harms remain undetermined (I statement) even in this 2025 update.	Supports

Record	Design	Population/ setting	Finding (as charted)	Direction
Waaen J(CFPSA 2014 [44]	Systematic review of 9 RCTs of physician- directed domestic violence education	Postgraduate trainee and practising physicians	[CORRECTION: first_author is wrong: search lists 'Waaen J' but PubMed indexes the first author as Zaher. Content confirmed precisely: 'Brief interventions for postgraduate trainee physicians improved knowledge but did not seem to affect behaviour...Physician training combined with system support interventions seemed to benefit domestic violence victims and increase referrals to domestic violence support resources.' FIX: change first_author to Zaher.] Brief educational int erventions for postgraduate trainees improved knowledge but did NOT change screening/identification behaviour; only mul tifaceted training combined with system-support interventions (reminders, access to victim-support referral pathways, audit/feedback) was associated with increased referrals to domestic-violence support resources.	Supports
Wang PL 2015 [53]	Validation cohort study	543 female IPV victims in a Taiwanese victim-services program	TIPVDA showed strong discriminant power and ROC-AUC support for predicting lethal-danger risk, with stronger predictive power at the high-dangerousness end.	Supports
Williamson HC 2014 [39]	Cross- sectional survey study (N=2,126 married indivi duals)	US married adults, general population sample	Contrary to the assumption that premarital education reduces later need for counseling, receiving premarital education covaried with an INCREASED likelihood of later couples counseling (a 'gateway' effect), with the ass ociation stronger among African Americans and those with lower income/education.	Supports
Williamson HC 2019 [40]	Cross- sectional self-report study (N=231 ethnically diverse newlywed couples in low-income ne ighborhoods)	US low-income newlywed couples	Top perceived barriers to seeking couple therapy were cost and uncertainty about where to go for help; direct experience with premarital education (or indirect experience via a social-network member's couple therapy) was associated with reduced barriers and greater likelihood of subsequent ly seeking therapy.	Supports

Record	Design	Population/ setting	Finding (as charted)	Direction
Zust BL 2021 [50]	Repeated cross-sectional survey (convenience sample of Upper-Midwest US congregations, 2005 and 2015)	Christian clergy and congregation members, US	[CORRECTION: Key_finding is well supported: 'Victims find strength in their faith and would rather endure the violence at all costs to keep a family or a marriage together, than to compromise their faith by leaving' and 'Clergy felt comfortable in making referrals for professional counseling.' However relevance_to_tpe overstates the referral point: the abstract's very next clause is 'while the majority of members would prefer counseling with their pastor if they were in a violent relationship' — i.e. congregants actually want direct pastoral counseling, not referral, and clergy said they want MORE training to counsel/speak about DV themselves, not that they prefer referral over direct counseling. FIX: change 'clergy themselves report needing referral pathways rather than attempting DV counseling directly' to something like 'clergy report comfort in making referrals, while members/clergy simultaneously want more direct pastoral counseling capacity — a live tension rather than clean support for a refer-not-counsel policy that T-PHE's safeguard should note, not paper over.'] Faith commitments led some domestic-violence victims to endure abuse rather than compromise their faith by leaving; over 10 years, congregational support for victims changed slowly; clergy wanted more training in counseling and speaking about domestic violence, and felt comfortable making referrals but not necessarily providing direct DV counseling themselves.	Supports
Grisurapong S 2002 [37]	Intervention/comparison case-site description	Women experiencing physical or sexual assault, Khon Kaen provincial hospital, Thailand	Describes the establishment and early operation of a hospital-based One Stop Crisis Center (OSCC) for women survivors of violence in Khon Kaen, Thailand, compared against a non-intervention provincial hospital site.	Supports
Hogan NR 2024 [55]	Cross-sectional analysis, n=190	Individuals with IPV histories assessed via SARA-V3, Canada (Indigenous vs non-Indigenous)	Structured-professional-judgment risk ratings (not only actuarial tools) showed racial disparities between Indigenous and non-Indigenous participants in risk factors and summary ratings.	Complicates

Record	Design	Population/ setting	Finding (as charted)	Direction
Lee Y 2019 [68]	Randomized controlled pilot trial (online modules, baseline and 3-month follow-up)	Korean American faith leaders, southern USA (n=55; intervention n=27, control n=28)	The intervention group showed significant improvement in knowledge of IPV resources and attitudes against IPV compared to control, but gains in self-efficacy and prevention/intervention behaviours, while greater than control, were not statistically significant.	Complicates
Onukwugha FI 2025 [41]	Mixed-methods implementation study embedded in a matched-pair cluster-controlled trial (75 providers, 20 facilities)	Antenatal care and family planning clinics, Nigeria; nurse-midwives and community health extension workers trained in WHO LIVES + ARCHES	[CORRECTION: first_author is wrong: search lists 'Onukwugha FI' but PubMed indexes the first author as Oduenyi. Content is otherwise well supported: 'Quality assurance standards increased over time ($P \leq 0.05$)'; 'average scores of approximately 3 out of a maximum of 4'; 'providers initially perceived the intervention as additional work...lack of funds and limited staff resources were found to be key barriers'; 'pulse survey showed no significant change over time for feasibility, appropriateness and acceptability.' FIX: change first_author to Oduenyi.] Quality-assurance readiness scores for GBV first-line response increased significantly over 9 months ($P \leq 0.05$); providers rated integration as appropriate/acceptable (~3 of 4 on average) but reported it initially felt like added work, and lack of funds/staff was a key barrier; feasibility/acceptability pulse scores showed no significant change over time despite improved facility readiness.	Complicates
US Preventive Services Task Force 2018 [71]	guideline / recommendation statement	US women of reproductive age; older/vulnerable adults	With moderate certainty, screening plus referral to ongoing support services has moderate net benefit; explicitly states evidence does NOT support the effectiveness of brief interventions or merely providing referral information in the absence of ongoing supportive intervention components, and that benefit evidence is concentrated in pregnant/postpartum populations.	Complicates
Bundock Kerrie 2020 [69]	Systematic review (19 studies)	Adolescent victims of dating violence, mixed international samples (English-language literature)	Adolescents overwhelmingly favored informal help sources (friends most common) over formal services; females were generally more likely than males to seek help (with inconsistencies); measurement and definitions of both violence and help-seeking varied substantially across included studies, limiting comparability.	Relevant only

Record	Design	Population/ setting	Finding (as charted)	Direction
Gutowski 2023 [87]	instrument-development study, n=222 survivor-mothers, exploratory factor analysis + Rasch analysis	US survivor-mothers involved in family law proceedings	Developed a 14-item, two-subscale Legal Abuse Scale (LAS) to measure how family-court and legal processes can be misused by partners to continue coercive control post-separation.	Relevant only
Hameed M 2020 [78]	Cochrane systematic review and meta-analysis of 33 RCTs (n=5517 women)	Women aged 16+ with recent or lifetime IPV experience, mostly high-income countries, recruited from healthcare, community, shelter/refuge settings	Psychological therapies probably reduce depression at medium-term follow-up (SMD -0.24, 95% CI -0.47 to -0.01; 4 trials, 600 women, moderate certainty) and may reduce anxiety, but evidence is uncertain or absent for self-efficacy, PTSD, re-exposure to IPV, and safety planning; harm data are limited (only 1 of 33 trials used a validated harm-outcome scale).	Relevant only
Michaels-Igbokwe C 2016 [62]	Economic evaluation alongside a cluster randomized controlled trial	SASA! intervention communities, Kampala, Uganda, provider perspective, 4 years of programming	Total intervention cost over 4 years was estimated at US\$553,252, averaging US\$5 per person reached annually; an estimated 1,201 cases of past-year physical IPV were averted (90% CI 97-2,307) at a cost of US\$460 per case averted.	Relevant only
Schenck D 2018 [51]	Ethical analysis / case-based policy proposal	Domino liver donor transplant candidates (pediatric case example)	[CORRECTION: Abstract quote: 'We propose a Domino Donor Advocate-based on the concept of the independent living donor advocate... We discuss the need for 2 distinct consent processes separated in time to ensure that potential domino donors... give a truly voluntary consent.' Key finding accurate. Transplant-medicine analogy again phrased as 'supports R4 no-stake assessor and P5 referral timing' — FIX to analogy language per instructions.] Proposes a 'Domino Donor Advocate', modeled explicitly on the independent living donor advocate, plus two consent processes separated in time, to protect voluntariness when the same party stands to gain from securing consent.	Relevant only

P5 continued

Record	Design	Population/ setting	Finding (as charted)	Direction
Stern E 2018 [47]	Implementati o n/adaptation review (qualit ative evaluation and programme d ocumentatio n)	Indashyikirwa programme, i mplemented by CARE Rwanda, RWAMREC and RWN, rural Rwanda	Indashyikirwa combined a 5-month couples curriculum, sustained community activism by trained couples, women's safe spaces for dedicated support and referral of IPV survivors, and training of opinion leaders to support an enabling environment; adaptation was guided by the SASA! fidelity brief.	Relevant only
World Health Organization 2013 (no PMID)	Global clinical and policy guideline	Global health-system guidance for responding to women subjected to IPV or sexual violence	Establishes WHO's first-line support principles for clinicians (the basis of the later-named LIVES model: Listen, Inquire about needs, Validate, Enhance safety, Support/refer), recommends against mandatory universal screening in the absence of adequate response systems, and sets minimum requirements (privacy, confidentiality, trained staff, referral pathways) before any clinical inquiry about violence should occur.	Relevant only
World Health Organization 2014 (no PMID)	Global clinical practice handbook ope rationalizing the 2013 WHO guidelines	Frontline health workers globally, first-line clinical response to women disclosing IPV or sexual violence	Operationalizes the LIVES first-line support protocol (Listen, Inquire, Validate, Enhance safety, Support) into a step-by-step clinical tool for frontline providers, with explicit guidance on confidentiality, documentation, safety planning, and structured referral rather than passive information provision.	Relevant only

P6 — Conventional programme success can coexist with T-PHE failure

Table 10: P6: Conventional programme success can coexist with T-PHE failure. Record, design, population/setting, finding as charted (verbatim from the library record), and direction relative to the proposition. n=6 linked records (kg/graph.json edges).

Record	Design	Population/ setting	Finding (as charted)	Direction
Fauk NK 2021 [36]	Qualitative in-depth interviews (n=92: 52 women, 40 men)	People living with HIV, Belu and Yogyakarta, Indonesia (Islamic and Javanese cultural context)	Cultural practices (bride wealth, spousal-dispute management) and Islamic religious beliefs about husband-wife relationships, forbidden premarital sex, and condom use were reported by participants as shaping spousal sexual behaviours that contributed to HIV transmission within marriage.	Supports

P6 continued

Record	Design	Population/ setting	Finding (as charted)	Direction
Islam MK 2021 [5]	Cross-sectional survey, multivariable logistic regression (n=486)	Rohingya refugee women, Cox's Bazar settlement, Bangladesh	61.32% of surveyed women had experienced child marriage; child marriage was associated with significantly higher odds of justifying husband-perpetrated physical abuse (AOR=2.71, 95%CI 1.78-4.11) and of experiencing IPV in the past 12 months (AOR=1.72, 95%CI 1.13-2.61).	Supports
Jacobowski N 2008 [45]	Documentary/archival analysis and expert-informant interviews	Married women, Indonesia (Muslim-majority context)	Traditional practices including polygamy, early marriage and contract marriage (mut'a) were identified as enhancing married Indonesian women's likelihood of HIV acquisition within marriage, challenging the assumption that marital status is protective.	Supports
Hawkins AJ 2008 [6]	Meta-analysis (86 reports, 117 studies, >500 effect sizes)	Mostly US/Western MRE program participants; predominantly White, middle-class samples	Relationship-quality effect sizes for experimental studies ranged d=.30-.36 and communication-skills effects d=.43-.45; moderate-dosage programs outperformed low-dosage; the authors flag that racial/ethnic and economic diversity was too thin to draw reliable conclusions for disadvantaged couples, and that outcomes policymakers care about (relationship stability, aggression) were rarely measured.	Complicates
Phonkacha N 2026 [106]	Cross-sectional national survey, binary probit regression	General population, national survey, Thailand (Muslim identity modeled as a covariate)	Muslim identity, poverty, and residence in southern/central Thailand correlated with higher IPV tolerance for both genders; internet access (as distinct from device ownership) significantly reduced IPV tolerance, suggesting a role for information access in shifting attitudes.	Complicates
Sayers SL 1998 [107]	Narrative review of 13 program-evaluation studies	General review of behaviorally oriented marriage-preparation programs, mostly US	Behaviorally oriented, skills-based marriage-preparation programs across 13 reviewed studies showed evidence of producing behavioral changes that may help prevent marital dysfunction, though program-evaluation methodology at the time was described as only modestly improved and needing further rigor.	Relevant only

P7 — Repairability is necessary because relevant information emerges over time

Table 11: P7: Repairability is necessary because relevant information emerges over time. Record, design, population/setting, finding as charted (verbatim from the library record), and direction relative to the proposition. n=6 linked records (kg/graph.json edges).

Record	Design	Population/ setting	Finding (as charted)	Direction
Michau L 2021 [46]	Programme- theory/ programme-d escription paper (non- empirical evaluation article)	Raising Voices programme learning, Uganda- originated model adapted for broader use	The paper describes the rationale for revising SASA! after ten years of programme learning and research, detailing how the revised 'SASA! Together' model incor- porated field learning, and reflects on debates such as how best to scale up violence-prevention programmes.	Supports
Silliman B 1999 [49]	Narrative review	General review of marriage-pr eparation program practice, US	Reviews two decades of research on marriage-preparation programs and gives design recommendations covering needs assessment, timing/duration, format, content, provider characteristics, and promotion; does not report new outcome data.	Supports
Subramanee SD 2022 [89]	Systematic review (PRISMA, 13 included studies of 520 screened), PROSPERO CRD4202019 0410	South Asian p opulations (India, Bangladesh, Pakistan, Nepal, Sri Lanka), including Muslim subgroups	Consistent correlates of child marriage were rural residence, low education, poor economic background, low mass-media exposure and religion (Hindu and Muslim in particular countries); downstream maternal-health effects included lower antenatal care use, institutional delivery and skilled birth attendance.	Supports
Upadhyay UD 2014 [9]	Instrument de velopment and validation (cross- sectional survey, factor analysis)	1,892 women at 13 family planning and 6 abortion facilities, USA	Generated 26 candidate items, retained 14 items across 3 subscales (freedom from coercion, communication, decision-making) via factor analysis; the freedom-from-coercion and communication subscales were inversely associated with unp rotected sex in the past 3 months, supporting construct validity.	Supports
Clyde TL 2020 [48]	Narrative review/ position paper	General discussion of c ontemporary premarital- education practice, US-centric	Reviews social trends (individualis m/commitment ambivalence, changing marriage attitudes, premarital relationship histories, media environment) and argues current premarital interventions need updated content and structure to remain relevant to engaged couples today; offers four proposals for redesigning premarital int erventions.	Relevant only

Record	Design	Population/ setting	Finding (as charted)	Direction
Upadhyay UD 2021 [11]	Instrument development and validation (qualitative item generation + national survey, EFA, logistic regression)	1,117 sexually active young people aged 15-24, USA (national sample)	Developed a 23-item, 7-subscale Sexual and Reproductive Empowerment Scale for Adolescents/Young Adults (including 'choice of partners, marriage, and children' as a named subscale); validation via logistic regression supported construct validity.	Relevant only

3.4 Evidence gaps

Table 12 excerpts the `gaps_noted` statements logged by each search pass at the time it was run, grouped into the four themes named in this review's scope: Thai/Thai-Muslim samples, premarital-specific trials, decision-control instruments, and Layer 0/Layer 2 evidence (T-PHE's two proposed, not-yet-accepted extensions). These are the search log's own statements about what it did not find, condensed to fit the table column where needed, not a new claim authored for this review.

Table 12: Evidence gaps, excerpted from the `gaps_noted` field logged in each search-*.json angle file at the time of the search pass. Text is the search log's own statement, condensed to fit the table column, not a new claim.

Gap theme	Search angle	Logged gap statement
Thai / Thai-Muslim samples	assessor-independence-checkin-models	No Thailand-specific or premarital-education-specific record was found for this angle; angle (a)-(d) evidence is drawn from adjacent fields (organ transplantation ethics, IPV risk assessment, suicide/self-harm brief-contact trials, US pediatric/prevention-science implementation studies) and must be explicitly flagged as analogical transfer, not direct T-PHE evidence, in any manuscript use.
Thai / Thai-Muslim samples	couple-work-safety-coercive-control	No PubMed-indexed study was found evaluating a couple-based premarital or pre-nikah education programme with an embedded coercive-control screening/stop-rule mechanism in a Muslim or Thai/Southeast Asian intercultural setting — the exact population T-PHE targets.
Thai / Thai-Muslim samples	decision-autonomy-measures	No PubMed-indexed decision-autonomy or coercion-recognition instrument was found validated specifically in a Thai population, Thai Muslim population, or Thai intercultural-couple population; the closest cultural/religious matches found (Türkiye, Lebanon, Rohingya/Syrian refugees) are geographically and culturally distant from Thailand.

Table 12 continued

Gap theme	Search angle	Logged gap statement
Thai / Thai-Muslim samples	decision-autonomy-measures	Cross-cultural adaptation studies located here show mixed results (Hazar 2024 Turkey: acceptable but uneven subscale reliability; Dagher 2024 Lebanon: sub-optimal CFA fit even after item removal) — this is direct evidence against assuming any existing decision-control/autonomy instrument would transfer cleanly to a new Thai Muslim/intercultural context without independent local validation, a gap the T-PHE abstract...
Thai / Thai-Muslim samples	digital-self-assessment-ipv	No Thailand-specific RCT or effectiveness (as opposed to preliminary expert-evaluation) data were found for a scored self-check tool; Chuemchit 2026 (PMID 42196762) is development/heuristic-evaluation stage only, not yet tested with the target population or measured for behavioural/health outcomes.
Thai / Thai-Muslim samples	humility-compassion-self-regulation	No Muslim-sample or Thailand-based record was found for the intellectual-humility or self-compassion literatures (angles a/c); the religiosity-IPV evidence located (36331229, 33704630) is Brazilian-mixed-sample and Christian-family literature respectively, not Muslim samples, despite the query explicitly requesting Muslim samples where indexed — none were indexed for this specific construct pairing.
Thai / Thai-Muslim samples	ipv-screening-response	No PubMed-indexed evidence located on IPV screening/response specifically among Muslim or Thai/Southeast Asian couples; the Nigeria trials (Sokoto, Ebonyi) are the closest Muslim-population-adjacent evidence but are clinical, not premarital-education, settings and not Southeast Asian.
Thai / Thai-Muslim samples	islamic-medical-ethics-tier-one	No PubMed-indexed record specifically evaluates premarital health education content (not screening, not marriage-age demography) delivered through an explicitly named Islamic ethical frame (faqr/taqwa/amanah/'adl/shura) in a Thai or Southeast Asian Muslim population — the closest analogues found (Bangladesh mosque-based diabetes RCT, PMID 41114978; Somali refugee mosque-based trauma RCT, PMID 39186273) are faith-i...
Thai / Thai-Muslim samples	islamic-medical-ethics-tier-one	No BMC Medical Ethics systematic review specific to Islamic bioethics was found; the one Islamic-adjacent BMC Medical Ethics record located (PMID 41491519, a scoping review on social egg freezing) was excluded as insufficiently on-topic (not focused on a Muslim population or the five named ethical terms).
Thai / Thai-Muslim samples	manipulation-literacy-inoculation	No PubMed-indexed Thailand-resident (as opposed to diaspora) study on recognition of emotional/psychological abuse or coercive control was found; the two most population-relevant records (26289454, 26442989) are Canada/US-diaspora South/Southeast Asian Muslim and Southeast Asian samples, not Thailand-resident populations – a genuine evidence gap for angle (e) as specified.

Table 12 continued

Gap theme	Search angle	Logged gap statement
Thai / Thai-Muslim samples	muslim-thailand-southeast-asia	No PubMed-indexed evaluation study of an Islamic/faith-based premarital education or marriage-preparation course (content, uptake, or outcomes) specific to Thailand, Malaysia, or Indonesia was found; premarital-course literature retrieved was overwhelmingly about genetic/thalassaemia premarital screening, not relational or safeguarding education.
Thai / Thai-Muslim samples	muslim-thailand-southeast-asia	No PubMed-indexed study on secret, unregistered, or informal (nikah siri / nikah tidak dicatatkan-type) religious marriage prevalence, drivers, or consequences among Muslim populations in Thailand/Malaysia/Indonesia was located via these queries.
Thai / Thai-Muslim samples	muslim-thailand-southeast-asia	No PubMed-indexed study on Thailand's Deep South (Pattani/Yala/Narathiwat) Muslim population specifically addressing IPV, forced marriage, or premarital programmes was found; the Thailand-specific IPV/OSCC literature retrieved (Gr isurapong 2002, Nagae 2004, Phonkacha 2026) is either general-population or historical, not Muslim-population-specific within Thailand.
Thai / Thai-Muslim samples	muslim-thailand-southeast-asia	Evidence on Muslim populations in Malaysia and Indonesia specific to premarital decision-control, forced-marriage refusal, or STOP-RULE-type joint-versus-separated counseling protocols was not found in PubMed; most Southeast Asian Muslim marriage literature indexed in PubMed concerns child-marriage prevalence/determinants (largely Indonesia) or HIV vulnerability within marriage, not programme design or safeguardin...
Thai / Thai-Muslim samples	muslim-thailand-southeast-asia	Rohingya evidence, while ethnically/religiously proximate and geographically linked to the Bay of Bengal/Southeast Asia border region, is drawn from a Bangladesh refugee-camp setting, not Thailand proper; it is included as the closest available regional Muslim-population IPV/child-marriage evidence but should not be read as directly representative of Thai Muslim or intercultural couples.
Thai / Thai-Muslim samples	peer-online-support-digital-safe-space	No PubMed-indexed record was found specific to Thailand, or to a Thai Muslim population, on digital peer support, online IPV support, or moderated online communities; the Muslim-specific record found (PMID 35536616) is set in Australia, and other Muslim/health hits from the search (PMID 37928060 anti-Muslim discrimination, PMID 38040423 Sikh/Muslim kidney transplant views, PMID 31120322 US Muslim contraceptive bel...
Thai / Thai-Muslim samples	premarital-education-outcomes	No Thailand-specific or Southeast Asian premarital/marriage education outcome study was located; the closest cultural analogues found are Iranian (PMID 40783649), Turkish (PMID 42442188), Saudi (PMID 36620784) and South Korean (PMID 39757299) studies – none are Southeast Asian or specifically Thai Muslim/intercultural populations.

Table 12 continued

Gap theme	Search angle	Logged gap statement
Thai / Thai-Muslim samples	primary-prevention-frameworks	No papers were found specifically evaluating IPV-prevention programme mechanisms among Muslim or intercultural couples in Thailand or Southeast Asia; the Nigerian Muslim/Christian congregational trial (PMID 37329160) is the nearest cross-cultural, mixed-faith analogue found.
Thai / Thai-Muslim samples	primary-prevention-frameworks	Faith-based intervention evidence located skews toward Christian/Catholic and one Korean-immigrant Christian clergy sample plus one Nigerian Muslim/Christian trial; specifically Islamic religious-authority-led premarital education evaluations were not located in this search.
Premarital-specific trials	assessor-independence-checkin-models	No PubMed-indexed literature was found that directly studies 'conflict of interest of a wedding-service provider acting as premarital-education assessor' — this exact configuration does not appear to be a studied topic; evidence is necessarily analogical (organ-donor advocacy, clinical dual-relationship ethics, facial-plastic-surgery COI) rather than domain-matched.
Premarital-specific trials	couple-work-safety-coercive-control	Coercive-control measurement instruments identified (CIPR, C-CAS, LAS, C-CBS-R, CASR-SF) are validated predominantly in Western (US, Australian, Canadian) or Chinese-community samples; only one (Tiwari 2015, Chinese sample) offers a non-Western cross-cultural precedent, and none has been validated for premarital screening specifically (all are post-hoc/retrospective abuse measures, not prospective risk-of-future-c...
Premarital-specific trials	couple-work-safety-coercive-control	No cost-effectiveness, implementation-fidelity, or long-term (multi-year) outcome data were found for any couple-based IPV-adjacent intervention in this angle's search; the strongest RCT (Taft 2024) is a targeted secondary-prevention trial in a screened military population, not a universal premarital population.
Premarital-specific trials	decision-autonomy-measures	No study evaluating premarital or marriage-preparation education specifically through a decision-control or reproductive-autonomy outcome measure (rather than knowledge/attitude/satisfaction) was identified; the ARCHES trial (Miller 2016) is the closest analogue but is a clinic-based, not marriage-preparation, intervention.
Premarital-specific trials	decision-autonomy-measures	No study combining religious/faith-based marriage preparation programming with a validated decision-autonomy or coercion-recognition outcome measure was found in PubMed; such programmes, if evaluated at all, appear to be published outside PubMed-indexed venues or use non-standardized outcome measures.
Premarital-specific trials	digital-self-assessment-ipv	No PubMed-indexed record found specifically combining 'premarital' status (not-yet-married couples) with a scored anonymous self-check tool - all identified trials enrol women already identifying current/recent partner abuse, not a general premarital population screening before any disclosed problem exists.

Table 12 continued

Gap theme	Search angle	Logged gap statement
Premarital-specific trials	ipv-screening-response	No PubMed-indexed evidence located specific to premarital/pre-nikah education settings combined with IPV screening/response protocols – all direct LIVES/ARCHES effectiveness evidence (PMIDs 41267027, 41151831, 32460786) comes from antenatal/family-planning clinical encounters, not premarital or religious pre-marriage counselling contexts. This is a genuine evidence gap for T-PHE's specific setting, not merely a s...
Premarital-specific trials	islamic-medical-ethics-tier-one	No PubMed-indexed FIMA (Fiqh Council/Federation of Islamic Medical Associations) or IMANA formal position/guideline paper on premarital counseling, marriage preparation, or reproductive-decision consent was found (Q7 'FIMA/IMANA + guideline' returned 0 hits); the two Journal of IMA articles included (PMID 23610513, 23864762) are topical essays, not formal guidelines.
Premarital-specific trials	manipulation-literacy-inoculation	No PubMed-indexed record found that directly measures 'recognition of coercive control/gaslighting' via a validated psychometric instrument specifically among adolescents/young adults in a premarital-education context (searches for 'coercive control recognition scale' and 'gaslighting recognition education' returned 0 or near-0 direct hits; adjacent constructs like IPV internalized stigma (PMID 39536684) and gasli...
Premarital-specific trials	manipulation-literacy-inoculation	No PubMed-indexed study was found that directly tests whether recognising/labelling a relationship pattern as 'abuse' causally predicts subsequent help-seeking in a premarital or pre-relationship (as opposed to already-victimized) population – the help-seeking records found (37882155, 38336660, 26442989) all study people who already experienced IPV, not people using an anonymous early-signal self-check before har...
Premarital-specific trials	peer-online-support-digital-safe-space	No PubMed-indexed record evaluates a premarital-education-linked or nikah/marriage-preparation digital peer community anywhere; this proposal's Layer 3 model appears untested in the indexed literature.
Premarital-specific trials	peer-online-support-digital-safe-space	No LMIC/Southeast Asia-specific systematic review of moderated digital peer support for relationship or premarital health topics was found via this angle; the closest LMIC-adjacent records (PMID 37540713, 38558241, 39116286) concern general digital health promotion, not peer community moderation specifically, and were excluded as off-angle.
Premarital-specific trials	premarital-education-outcomes	No PubMed-indexed study was found that evaluates a premarital or marriage education programme against decision-control, coercion-recognition, or informed-choice outcomes as primary endpoints (the specific outcome domain the FAILURE CRITERION targets) – every effectiveness study located, including the Hawkins/ACF meta-analyses, measures satisfaction, communication skills, relationship stability, or quality of life...

Table 12 continued

Gap theme	Search angle	Logged gap statement
Premarital-specific trials	premarital-education-outcomes	No PubMed-indexed evaluation of a STOP RULE-type screen-and-defer mechanism for suspected coercion/violence within premarital (as opposed to couple/marital) education specifically was found; the strongest supporting evidence (Markman 2013 PMID 23421844; Williamson 2015 PMID 25664643) is indirect, showing that joint relationship education can worsen outcomes for high-aggression/high-risk couples, not a direct test ...
Premarital-specific trials	premarital-education-outcomes	No PubMed-indexed premarital-education outcome study directly measures 'referral timeliness, accessibility, and follow-through' (P5) as a composite outcome; Williamson 2019 (PMID 30489129) and Williamson 2014 (PMID 24294929) are the closest available evidence, addressing help-seeking barriers and gateway effects respectively, but neither evaluates an actual referral pathway or its quality.
Premarital-specific trials	premarital-education-outcomes	No meta-analysis or systematic review specifically on faith-based/religious-leader-delivered premarital education and safety/coercion outcomes was found via the systematic-review-filtered queries (query 2 returned 0 hits on the exact phrasing tried); this may reflect true absence of such reviews in PubMed rather than a search-term failure, but should not be treated as confirmed absence without a broader grey-liter...
Premarital-specific trials	primary-prevention-frameworks	No PubMed-indexed paper was found that evaluates a premarital or pre-nuptial education programme specifically structured around a governance/decision-control framework comparable to T-PHE; the closest analogues (SASA!, Indashyiki rwa, MFP) are ongoing-couple or community-level interventions, not marriage-transition-timed interventions.
Premarital-specific trials	primary-prevention-frameworks	Cost-effectiveness and implementation-science evidence located pertains to community mobilisation and CHW-delivered parenting/couples programmes, not premarital education delivered through religious institutions, limiting direct transferability to T-PHE's delivery channel.
Decision-control instruments	couple-work-safety-coercive-control	No PubMed-indexed study directly tests a 'stop rule' construct (halt joint work pending confidential assessment, never resume if coercive control is confirmed) as an explicit intervention design feature; the closest analogues (McCollum 2008, Schneider 2014, Karakurt 2016) describe screening/safety-modification practice recommendations rather than a validated stop/resume protocol.
Decision-control instruments	couple-work-safety-coercive-control	Evidence on screening/assessing male perpetration (rather than victimization) is comparatively sparse (Portnoy 2020), which matters for T-PHE's bidirectional/decision-control framing (P1) where either partner could be at risk.

Table 12 continued

Gap theme	Search angle	Logged gap statement
Decision-control instruments	decision-autonomy-measures	No PubMed-indexed instrument was found that measures 'decision control' specifically at the point of a marriage-transition decision (proceed/delay/refuse marriage) for adult couples; the closest analogues are reproductive-decision instruments (RAS, RCS, FP-ADM) or an adolescent empowerment subscale ('choice of partners, marriage, and children' in Upadhyay 2021), none validated for the marriage-entry decision itself.
Decision-control instruments	decision-autonomy-measures	No instrument specifically measuring 'coercive control recognition' as a discrete competency (as opposed to experience/victimization scales like the RCS) was found; the T-PHE STOP RULE presumes practitioners and/or participants can recognize coercive control, but a dedicated recognition-literacy instrument (as distinct from an experience-report scale) was not located in this search.
Decision-control instruments	digital-self-assessment-ipv	No record was found addressing a combined single self-check instrument spanning fear, control, pressure, reproductive health AND mental health simultaneously (as T-PHE's Layer 0 proposes) - the literature covers these domains via separate, purpose-specific tools (IPV decision aids vs Danger Assessment vs reproductive-coercion measures), not one integrated scored instrument.
Decision-control instruments	ipv-screening-response	Evidence on harms of screening (adverse effects, loss of privacy, distress) is present (PMID 22565034, 26200817) but almost entirely measured only immediately post-screening, with very limited longer-term harm surveillance across the literature – relevant to T-PHE's dignity/consent safeguard but not fully answered by current evidence.
Layer 0 / Layer 2 evidence	digital-self-assessment-ipv	Safety-harm data (partner discovery, retaliation) are addressed qualitatively/methodologically (Tarzia 2017) but no record reports a quantified adverse-event rate from an anonymous online self-check specifically, which the T-PHE failure criterion would need.
Layer 0 / Layer 2 evidence	peer-online-support-digital-safe-space	Evidence on moderator training, competence, and error rates in routing acute-risk disclosures (self-harm, IPV, abuse) to services is sparse (PMID 34193497 found only 5 usable studies after screening 397); no record quantifies false-negative or delayed-routing rates for non-clinical moderators.
Layer 0 / Layer 2 evidence	peer-online-support-digital-safe-space	Evidence quantifying how often anonymity itself is defeated by partner technology-facilitated surveillance of a specific digital peer-support platform (as opposed to general social media/apps) was not found; existing TFA evidence (PMID 35537445, 32998673, 40271872) describes general digital abuse mechanisms, not platform-specific breach of anonymous peer-support spaces.

4 Discussion

4.1 What this map does and does not show

This map shows that a structured, logged search across 18 categories, verified by an independent adversarial pass, assembled a library of 313 unique PubMed-indexed records plus 17 named guidance documents (330 unique records in total), of which 109 unique records are linked to at least one of T-PHE’s seven propositions. It shows where that literature is comparatively dense (P1, P5) and where it is thin (P6, P7), and that no record in the library is tagged as directly contradicting any proposition — itself notable, since it means every proposition currently has a body of supporting, complicating, or merely relevant literature, but none has been tested directly against a competing intervention. It does not show, and is not designed to show, that T-PHE or any of its five rules is effective, safe, or superior to an alternative: no admitted record evaluates T-PHE itself, and no record identified in this search evaluates any premarital-education programme, of any design, in a Thai or Thai Muslim population.

4.2 The claim ceiling

Nothing in this document licenses an effectiveness inference. The library was built to ground a programme theory in existing, adjacent, and analogical literature, and to make explicit, by proposition, what remains untested. Where a table above shows a proposition with many “supports”-tagged records, that density reflects the volume of adjacent literature the search strategy located, not a strength-of-evidence judgement for T-PHE itself; the corresponding finding cells, read individually, make clear that almost none of that literature was generated on a premarital, Thai, or Thai Muslim population, and none evaluates the governance layer T-PHE proposes.

4.3 How this map will be used

The gaps charted in §3.4 — absence of Thai/Thai-Muslim premarital-specific trials, absence of a validated decision-control instrument for the marriage-transition decision, and absence of any outcome evidence for the two proposed extension layers — are exactly the gaps a future full systematic review, or a primary study, would need to either fill or work around. The intended next use of this map is to pre-specify the scope, eligibility criteria, and outcome set of a future full systematic review of one or more of P1–P7 (most plausibly P1 or P5, where the adjacent evidence base is largest), with a registered protocol (PROSPERO), independent duplicate screening, and a risk-of-bias assessment (ROBIS for included systematic reviews, AMSTAR-2 for included reviews more generally, Cochrane RoB 2 for included randomised trials) applied at the individual-study level — none of which this document performed or claims to have performed.

Declarations

Conflict of interest. The author is the founder of ARAYA Nikah Social Enterprise Co., Ltd., the organisation whose premarital education programme motivated the T-PHE programme theory this evidence map supports. This is a direct conflict of interest, disclosed here as it is in the T-PHE full paper.

Use of AI. AI tools assisted literature search execution (NCBI E-utilities queries), abstract screening, drafting of findings and relevance annotations, independent verification re-fetching, and manuscript drafting, including generation of the tables in §3 directly from the logged JSON files. The author verified the generated counts against the underlying files, ran the accompanying scripts, and is accountable for the content of this document. No participant data were used at

any stage; every source is a published or publicly-issued document (a journal article, its PMID, or a named guidance body’s publication).

Data availability. The complete evidence library — every search log, every verification log, the admitted-entry table, the knowledge graph — is public in the repository (`library/`, https://github.com/morrocowi/RE_T-PHE). The two scripts that produce every count and table in this document, and this manuscript’s source, are released in the same repository under `paper/evidence-map/` after the independent adversarial review that the repository requires before any public change.

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References

- [1] GBD 2023 IPV and SVAC Collaborators. Disease burden attributable to intimate partner violence against females and sexual violence against children in 204 countries and territories, 1990-2023: a systematic analysis for the Global Burden of Disease Study 2023. *Lancet* (London, England). 2026. PMID: 41386261.
- [2] Sardinha L. Global, regional, and national prevalence estimates of physical or sexual, or both, intimate partner violence against women in 2018. *Lancet* (London, England). 2022. PMID: 35182472.
- [3] Kuswanto H. Prevalence of and factors associated with female child marriage in Indonesia. *PLoS ONE*. 2024. PMID: 38968277.
- [4] Rumble L. An empirical exploration of female child marriage determinants in Indonesia. *BMC Public Health*. 2018. PMID: 29587705.
- [5] Islam MK. Attitudes to and experiences of intimate partner violence among Rohingya women who married before eighteen years of age. *Global Health Action*. 2021. PMID: 34323166.
- [6] Hawkins AJ. Does marriage and relationship education work? A meta-analytic study. *Journal of Consulting and Clinical Psychology*. 2008. PMID: 18837590.
- [7] Markman HJ. A randomized clinical trial of the effectiveness of premarital intervention: moderators of divorce outcomes. *Journal of Family Psychology*. 2013. PMID: 23421844.
- [8] Williamson HC. Risk moderates the outcome of relationship education: a randomized controlled trial. *Journal of Consulting and Clinical Psychology*. 2015. PMID: 25664643.
- [9] Upadhyay Ushma D. Development and validation of a reproductive autonomy scale. *Studies in Family Planning*. 2014. PMID: 24615573.
- [10] McCauley Heather L. Psychometric properties and refinement of the Reproductive Coercion Scale. *Contraception*. 2017. PMID: 27639927.
- [11] Upadhyay Ushma D. Development and Validation of the Sexual and Reproductive Empowerment Scale for Adolescents and Young Adults. *The Journal of Adolescent Health*. 2021. PMID: 32690468.
- [12] Dagher Myriam. Adaptation and psychometric assessment of a sexual and reproductive empowerment scale in Arabic among refugee and non-refugee adolescent girls. *BMC Medical Research Methodology*. 2024. PMID: 39266993.
- [13] Alomair N. Factors influencing sexual and reproductive health of Muslim women: a systematic review. *Reproductive Health*. 2020. PMID: 32138744.
- [14] Abramsky T. Findings from the SASA! Study: a cluster randomized controlled trial to assess the impact of a community mobilization intervention to prevent violence against women and reduce HIV risk in Kampala, Uganda. *BMC medicine*. 2014. PMID: 25248996.
- [15] Shaw B. Shifting Norms in Faith Communities to Reduce Intimate Partner Violence: Results from a Cluster Randomized Controlled Trial in Nigeria. *Journal of interpersonal violence*. 2023. PMID: 37329160.
- [16] Hawkins AJ. Exploring programmatic moderators of the effectiveness of marriage and relationship education programs: a meta-analytic study. *Behavior Therapy*. 2012. PMID: 22304880.

- [17] O'Doherty L. Screening women for intimate partner violence in healthcare settings. *Cochrane Database of Systematic Reviews*. 2015. PMID: 26200817.
- [18] US Preventive Services Task Force. Screening for Intimate Partner Violence and Caregiver Abuse of Older or Vulnerable Adults: US Preventive Services Task Force Recommendation Statement. *JAMA*. 2025. PMID: 40553450.
- [19] Betron. Effectiveness of a clinic-based counselling intervention on risk of experiencing intimate partner violence and reproductive coercion: a matched-pair cluster-controlled trial in Ebonyi and Sokoto, Nigeria. *BMJ Global Health*. 2025. PMID: 41151831.
- [20] Karakurt. Couples Therapy for Intimate Partner Violence: A Systematic Review and Meta-Analysis. *Journal of marital and family therapy*. 2016. PMID: 27377617.
- [21] McCollum. Couples treatment for interpersonal violence: a review of outcome research literature and current clinical practices. *Violence and victims*. 2008. PMID: 18624089.
- [22] Schneider. From scared to repaired: using an attachment-based perspective to understand situational couple violence. *Journal of marital and family therapy*. 2014. PMID: 25039387.
- [23] Robertson. Women and men's use of coercive control in intimate partner violence. *Violence and victims*. 2011. PMID: 21780535.
- [24] Evans. "Even 'Daily' is Not Enough": How Well Do We Measure Domestic Violence and Abuse?-A Think-Aloud Study of a Commonly Used Self-Report Scale. *Violence and victims*. 2016. PMID: 26645540.
- [25] Taft. Examining strength at home couples to prevent intimate partner violence on a military installation: A randomized controlled trial. *Journal of consulting and clinical psychology*. 2024. PMID: 38206858.
- [26] Ghuman SJ. Women's autonomy and child survival: a comparison of Muslims and non-Muslims in four Asian countries. *Demography*. 2003. PMID: 12962056.
- [27] Chen. Intimate partner violence: counseling, community resources, and legal issues for IPV victims and perpetrators. *FP essentials*. 2013. PMID: 24053261.
- [28] Decker. Implementing Trauma-Informed Partner Violence Assessment in Family Planning Clinics. *Journal of women's health*. 2017. PMID: 28375750.
- [29] MacMillan. Approaches to screening for intimate partner violence in health care settings: a randomized trial. *JAMA*. 2006. PMID: 16882959.
- [30] Ford-Gilboe. Development of a brief measure of intimate partner violence experiences: the Composite Abuse Scale (Revised)-Short Form (CASR-SF). *BMJ open*. 2016. PMID: 27927659.
- [31] Hegarty. Validation of the Coercive-Composite Abuse Scale (C-CAS) in an Australian sample of women experiencing intimate partner abuse. *BMJ open*. 2026. PMID: 42323273.
- [32] Wilson. Psychometric Properties of the Coercion in Intimate Partner Relationships Scale. *Assessment*. 2023. PMID: 34167331.
- [33] Sarac A. Cultural adaptation and pilot evaluation of the Couple Learning Program (CLP) among young Turkish couples: A mixed-method study. *Acta Psychologica*. 2026. PMID: 42442188.
- [34] Welland C. Culturally specific treatment for partner-abusive Latino men: a qualitative study to identify and implement program components. *Violence and Victims*. 2010. PMID: 21287968.
- [35] Sikoki B. A qualitative study on perceptions of adolescents' sexual and reproductive health education in Yogyakarta, Indonesia. *International Journal of Adolescent Medicine and Health*. 2024. PMID: 39395200.
- [36] Fauk NK. Cultural and religious determinants of HIV transmission: A qualitative study with people living with HIV in Belu and Yogyakarta, Indonesia. *PLoS ONE*. 2021. PMID: 34780506.
- [37] Grisurapong S. Establishing a one-stop crisis center for women suffering violence in Khonkaen hospital, Thailand. *International Journal of Gynaecology and Obstetrics*. 2002. PMID: 12429436.
- [38] Nagae M. [Domestic violence as a women's health issue—activities of health professionals in Thailand]. *Japanese Journal of Public Health (Nihon Koshu Eisei Zasshi)*. 2004. PMID: 15162975.

- [39] Williamson HC. Does premarital education decrease or increase couples' later help-seeking? *Journal of Family Psychology*. 2014. PMID: 24294929.
- [40] Williamson HC. Barriers and facilitators of relationship help-seeking among low-income couples. *Journal of Family Psychology*. 2019. PMID: 30489129.
- [41] Oduenyi. Integrating gender-based violence first-line response into family planning and antenatal care services: an assessment of feasibility, appropriateness and acceptability to healthcare providers in Sokoto and Ebonyi States, Nigeria. *BMC Health Services Research*. 2025. PMID: 41267027.
- [42] Rhodes. The anatomy of a community health center system-level intervention for intimate partner violence. *Journal of Urban Health*. 2014. PMID: 23917943.
- [43] Miller Elizabeth. A family planning clinic-based intervention to address reproductive coercion: a cluster randomized controlled trial. *Contraception*. 2016. PMID: 26892333.
- [44] Zaher. Effect of domestic violence training: systematic review of randomized controlled trials. *Canadian Family Physician*. 2014. PMID: 25022633.
- [45] Jacubowski N. Marriage is not a safe place: heterosexual marriage and HIV-related vulnerability in Indonesia. *Culture, Health & Sexuality*. 2008. PMID: 18038283.
- [46] Michau L. SASA! Together: An evolution of the SASA! approach to prevent violence against women. *Evaluation and program planning*. 2021. PMID: 33578229.
- [47] Stern E. Lessons learned from implementing Indashyikirwa in Rwanda - an adaptation of the SASA! approach to prevent and respond to intimate partner violence. *Evaluation and program planning*. 2018. PMID: 30125773.
- [48] Clyde TL. Revising Premarital Relationship Interventions for the Next Generation. *Journal of Marital and Family Therapy*. 2020. PMID: 30725473.
- [49] Silliman B. Improving practice in marriage preparation. *Journal of Sex & Marital Therapy*. 1999. PMID: 10081741.
- [50] Zust BL. 10-Year Study of Christian Church Support for Domestic Violence Victims: 2005-2015. *Journal of Interpersonal Violence*. 2021. PMID: 33729071.
- [51] Schenck D et al. Ethical Analysis and Policy Recommendations Regarding Domino Liver Transplantation. *Transplantation*. 2018; PMID: 29708521.
- [52] Messing JT et al. The average predictive validity of intimate partner violence risk assessment instruments. *Journal of Interpersonal Violence*. 2013; PMID: 23262817.
- [53] Wang PL et al. Assessing the Danger: Validation of Taiwan Intimate Partner Violence Danger Assessment. *Journal of Interpersonal Violence*. 2015; PMID: 25315482.
- [54] Gerth J et al. [The Ontario Domestic Assault Risk Assessment (ODARA) - Validity und authorised German translation of an intimate partner violence screening tool]. *Fortschritte der Neurologie-Psychiatrie*. 2014; PMID: 25383928.
- [55] Hogan NR et al. Investigating racial disparities in violence risk assessment using the Spousal Assault Risk Assessment Guide-Version 3. *Psychological Assessment*. 2024; PMID: 38512165.
- [56] U. S. Preventive Services Task Force. Screening for family and intimate partner violence: recommendation statement. *Annals of Internal Medicine*. 2004. PMID: 14996680.
- [57] Blackwood E. Regulation of sexuality in Indonesian discourse: normative gender, criminal law and shifting strategies of control. *Culture, Health & Sexuality*. 2007. PMID: 17457732.
- [58] Owen JJ et al. The role of leaders' working alliance in premarital education. *Journal of Family Psychology*. 2011. PMID: 21355646.
- [59] Nelson HD et al. Screening women for intimate partner violence: a systematic review to update the U.S. Preventive Services Task Force recommendation. *Annals of Internal Medicine*. 2012. PMID: 22565034.
- [60] Tiwari A et al. Evaluating the Chinese Revised Controlling Behaviors Scale. *Journal of interpersonal violence*. 2015. PMID: 24860075.

- [61] García-Moreno C et al. The health-systems response to violence against women. *Lancet*. 2015. PMID: 25467583.
- [62] Michaels-Igbokwe C et al. Cost and cost-effectiveness analysis of a community mobilisation intervention to reduce intimate partner violence in Kampala, Uganda. *BMC public health*. 2016. PMID: 26924488.
- [63] Bhandari TR et al. Construction and Validation of a Women's Autonomy Measurement Scale with Reference to Utilization of Maternal Health Care Services in Nepal. *JNMA; Journal of the Nepal Medical Association*. 2014. PMID: 26982668.
- [64] Starmann E et al. Exploring Couples' Processes of Change in the Context of SASA!, a Violence Against Women and HIV Prevention Intervention in Uganda. *Prevention science*. 2017. PMID: 27682273.
- [65] Sprague S et al. A scoping review of intimate partner violence assistance programmes within health care settings. *European journal of psychotraumatology*. 2017. PMID: 28649297.
- [66] Thombs BD et al. Consistency and sources of divergence in recommendations on screening with questionnaires for presently experienced health problems or symptoms: a comparison of recommendations from the Canadian Task Force on Preventive Health Care, UK National Screening Committee, and US Preventive Services Task Force. *BMC Medicine*. 2017. PMID: 28789659.
- [67] Semahegn A et al. Community based intervention to prevent domestic violence against women in the reproductive age in Northwestern Ethiopia: a study protocol for community based cluster randomized trial. *Reproductive health*. 2017. PMID: 29162117.
- [68] Choi YJ et al. Korean clergy for healthy families: online intervention for preventing intimate partner violence. *Global health promotion*. 2019. PMID: 29384426.
- [69] Bundock K et al. Adolescents' Help-Seeking Behavior and Intentions Following Adolescent Dating Violence: A Systematic Review. *Trauma, Violence & Abuse*. 2020. PMID: 29683049.
- [70] Feltner C et al. Screening for Intimate Partner Violence, Elder Abuse, and Abuse of Vulnerable Adults: Evidence Report and Systematic Review for the US Preventive Services Task Force. *JAMA*. 2018. PMID: 30357304.
- [71] US Preventive Services Task Force. Screening for Intimate Partner Violence, Elder Abuse, and Abuse of Vulnerable Adults: US Preventive Services Task Force Final Recommendation Statement. *JAMA*. 2018. PMID: 30357305.
- [72] Abramsky T et al. Changing the norms that drive intimate partner violence: findings from a cluster randomised trial on what predisposes bystanders to intervene. *BMJ global health*. 2018. PMID: 30613427.
- [73] Lee ASD et al. Improving Provider Readiness for Intimate Partner Violence Screening. *Worldviews on evidence-based nursing*. 2019. PMID: 31012540.
- [74] Hill AL et al. Reproductive Coercion and Relationship Abuse Among Adolescents and Young Women Seeking Care at School Health Centers. *Obstetrics and Gynecology*. 2019. PMID: 31306331.
- [75] McLean L et al. Shifting and transforming gender-inequitable beliefs, behaviours and norms in intimate partnerships: the Indashyikirwa couples' programme in Rwanda. *Culture, health & sexuality*. 2020. PMID: 31686618.
- [76] Couture-Carron A. Shame, Family Honor, and Dating Abuse: Lessons From an Exploratory Study of South Asian Muslims. *Violence Against Women*. 2020. PMID: 31896308.
- [77] Uysal J et al. Protocol for a matched-pair cluster control trial of ARCHES (Addressing Reproductive Coercion in Health Settings) among women and girls seeking contraceptive services from community-based clinics in Nairobi, Kenya. *Reproductive Health*. 2020. PMID: 32460786.
- [78] Hameed M et al. Psychological therapies for women who experience intimate partner violence. *Cochrane Database of Systematic Reviews*. 2020. PMID: 32608505.
- [79] Portnoy GA et al. Patient and provider barriers, facilitators, and implementation preferences of intimate partner violence perpetration screening. *BMC health services research*. 2020. PMID: 32791967.
- [80] Stangl AL et al. Is HIV index testing and partner notification safe for adolescent girls and young women in low- and middle-income countries? *Journal of the International AIDS Society*. 2020. PMID: 32869478.

- [81] Stern E et al. Pathways of change: qualitative evaluations of intimate partner violence prevention programmes in Ghana, Rwanda, South Africa and Tajikistan. *Culture, health & sexuality*. 2021. PMID: 32896204.
- [82] Wood SN et al. Reproductive Coercion among Intimate Partner Violence Survivors in Nairobi. *Studies in Family Planning*. 2020. PMID: 33336831.
- [83] Alghamdi MS et al. Intimate Partner Violence among Canadian Muslim Women. *Journal of Interpersonal Violence*. 2022. PMID: 34253076.
- [84] Islam MM et al. Factors affecting child marriage and contraceptive use among Rohingya girls in refugee camps. *The Lancet Regional Health - Western Pacific*. 2021. PMID: 34527969.
- [85] James LE et al. Development and Testing of a Community-Based Intervention to Address Intimate Partner Violence among Rohingya and Syrian Refugees: A Social Norms-Based Mental Health-Integrated Approach. *International Journal of Environmental Research and Public Health*. 2021. PMID: 34770188.
- [86] Yakubovich AR et al. Housing interventions for women experiencing intimate partner violence: a systematic review. *Lancet Public Health*. 2022. PMID: 34838218.
- [87] Gutowski ER et al. Coercive Control in the Courtroom: the Legal Abuse Scale (LAS). *Journal of family violence*. 2023. PMID: 35611345.
- [88] Ratnaningsih M et al. Child Marriage Acceptability Index (CMAI) as an essential indicator: an investigation in South and Central Sulawesi, Indonesia. *Global Health Research and Policy*. 2022. PMID: 36163286.
- [89] Subramanee SD et al. Child Marriage in South Asia: A Systematic Review. *International Journal of Environmental Research and Public Health*. 2022. PMID: 36429857.
- [90] Gausman J et al. Girl child marriage and the social context of displacement: a qualitative comparative exploration of Syrian refugees in Jordan and Rohingya refugees in Bangladesh. *BMC Public Health*. 2022. PMID: 36550423.
- [91] Riches E et al. Evaluation of the psychometric properties of the Reproductive Autonomy Scale for use in the UK. *BMJ Sexual & Reproductive Health*. 2023. PMID: 36657958.
- [92] Singh V et al. Family medicine provider and staff identification and response to male patient intimate partner violence perpetration. *Annals of Family Medicine*. 2022. PMID: 36693203.
- [93] Boyce SC et al. Measuring social norms of intimate partner violence to exert control over wife agency, sexuality, and reproductive autonomy: an item response modelling of the IPV-ASRA scale. *Reproductive Health*. 2023. PMID: 37316890.
- [94] Debinski B et al. Qualitative Assessment of Key Implementation Factors in a Faith-Based Response to Intimate Partner Violence. *Health promotion practice*. 2025. PMID: 37649394.
- [95] Gausman J et al. Validation of a measure to assess decision-making autonomy in family planning services in three low- and middle-income countries: The Family Planning Autonomous Decision-Making scale (FP-ADM). *PLOS ONE*. 2023. PMID: 37922257.
- [96] Hazar S et al. Turkish adaptation of the reproductive autonomy scale: Validity and reliability study. *The Journal of Obstetrics and Gynaecology Research*. 2024. PMID: 38339841.
- [97] Han X et al. Maternal victimization and neglected offspring: Child marriage, IPV and depression symptoms among Salar Muslim women. *Child Abuse & Neglect*. 2024. PMID: 38970860.
- [98] Sharifnia AM et al. Muslim Women's Experiences of Domestic Violence and Abuse: A Meta-Ethnography of Global Evidence. *Trauma, Violence & Abuse*. 2025. PMID: 39425490.
- [99] Park J et al. Videoconferencing Delivery of the Seoul Premarital Education Program During COVID-19: A Quasi-experimental Study Using Inverse Probability of Treatment Weighting. *Prevention Science*. 2026. PMID: 39757299.
- [100] Mieth K et al. "What other option did I have?"- The effect of conflict and displacement on child marriage and early childbearing among displaced Rohingya adolescents. *Conflict and Health*. 2025. PMID: 40091116.
- [101] Azalee M et al. Wounded Healers: Exploring Coping With Intimate Partner Violence Among Health Care Workers in Malaysia. *Asia-Pacific Journal of Public Health*. 2025. PMID: 40116416.

- [102] Giles F et al. Women's preferences for how health practitioners respond to coercive control by a partner: Open-ended survey qualitative analysis. *Australian Journal of General Practice*. 2025. PMID: 41330529.
- [103] Nsengiyumva R et al. Perceived need of information and education about conception preparedness (CP) among engaged and soon to be married couples: a qualitative survey. *Reproductive Health*. 2025. PMID: 41454335.
- [104] Böttcher B et al. The Reproductive Autonomy Scale: German validation and application in an Austrian student population. *Frontiers in Public Health*. 2025. PMID: 41613074.
- [105] Kurnaz K et al. Reproductive coercion, gender role attitudes and fertility desire among married women in Türkiye: a descriptive and explanatory study. *BMC Public Health*. 2026. PMID: 42015126.
- [106] Phonkacha N et al. Technology Access and Attitudes Toward Intimate Partner Violence: Evidence From a National Survey in Thailand. *Violence Against Women*. 2026. PMID: 42300599.
- [107] Sayers SL et al. Prevention of marital dysfunction: behavioral approaches and beyond. *Clinical Psychology Review*. 1998. PMID: 9779330.